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This instruction implements Department of the Air Force Policy Directive (DAFPD) 48-1, *Aerospace and Operational Medicine Enterprise (AOME)*. This instruction applies to all active-duty Air Force Special Operations Command (AFSOC) operational medical personnel, pararescuemen (PJ), and Special Tactics Officer (STO) personnel. This instruction applies to AFSOC gained Air Force Reserve Command (AFRC) and Air National Guard (ANG) units, where "MDG" or "MTF" may be replaced with the applicable level of medical unit. Further, consider any exceptions for Air Reserve Components (ARC) found in Air Force (AF) guidance (e.g. Air Force Instruction (AFI) 48-101, *Aerospace Medicine Enterprise* and Air Force Manual (AFMAN) 48-149 *Flight and Operational Medicine Program (FOMP)*) applicable in this document. This instruction does not apply to the USSF, and units will comply with this instruction in applicable areas. This publication requires the collection and/or maintenance of information protected by the Privacy Act of 1974 authorized by Title 10 U.S.C., Sec 9013, Secretary of the Air Force. The applicable System of Records Notice (SORN) F033 AF D, Automated Orders Data System, available at: **https://dpcld.defense.gov/privacy/SORNS.aspx** AFSOC operational medical personnel are defined as medical personnel assigned to AFSOC medical unit type codes (UTCs), as well as other specific medical personnel assigned to AFSOC units (i.e. special tactics medical personnel). Ensure that all records created as a result of processes prescribed in this publication are maintained in accordance with (IAW) AFI 33-322, *Records Management and Information Governance Program*, and disposed of IAW Air Force Records Information Management System (AFRIMS) Records Disposition Schedule (RDS).". Refer recommended changes and questions about this publication to the Office of Primary Responsibility (OPR) using the Department of the Air Force (DAF) Form 847, *Recommendation for Change of Publication*; route DAF Forms 847

from the field through the appropriate functional chain of command. This publication may be supplemented at any level with OPR approval. Requests for waivers must be submitted to the OPR listed above for consideration and approval. The authorities to waive wing/unit level requirement in this publication are identified with a Tier ("T-0, T-2,or T-3") number following the compliance statement. Requests for waivers must be submitted through the chain of command to the appropriate Tier waiver approval authority IAW DAFMAN 90-161, *Publishing Processes and Procedures.* Requests for waivers of non-tiered items must be processed through command channels to the publication OPR for consideration. Compliance with attachments is mandatory. The use of the name or mark of any specific manufacturer, commercial product, commodity, or service in this publication does not imply endorsement by the Air Force.

SUMMARY OF CHANGES

This document has been substantially revised and must be completely reviewed. Major changes and updates include: use of Operational Clinical Services (OCS) rather than Acute Care Clinic (ACC), description of Special Operations Wing (SOW)/ Medical Operations Division (A3M) operations post elimination of Operational Support Medical (OSM) flights, clarification of expectations all AFSOC providers will participate in peer review, no use of medication dispensing devices outside medical treatment facilities, requirements for off duty employment, how operational medical personnel will address vendor requests for sponsorship, how to address use of non-Food and Drug Administration (FDA) approved products, and appropriate documentation of care rendered by pararescumen.

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Chapter 1

FUNCTIONAL AREA RESPONSIBILITIES

1.1. Introduction and Overview. This Instruction highlights responsibilities at wing level and below and establishes procedures for all AFSOC operational medical personnel described in Air Force Tactics, Techniques and Procedures (AFTTP) 3-42.6, *USAF Medical Support for Special Operations Forces*. AFSOC operational medical personnel, or "operational medics," are defined as medical Air Force Specialty Codes (AFSC) assigned to an AFSOC UTC outside of the Air Force Medical Command (AFMEDCOM)-aligned medical treatment facilities (MTFs) and Air Reserve Component medical units. PJ personnel are not considered operational medical personnel as they have operational requirements outlined in their combatant AFSCs.

1.2. Waiver Authority. Requests for waivers must be submitted through the chain of command to the appropriate Tier waiver approval authority.

1.3. Organizational Responsibilities.

1.3.1. AFSOC Command Surgeon (SG). The AFSOC/SG is responsible for establishing, coordinating, and sustaining a medically ready AFSOC force and for organizing, training, equipping, and validating AFSOC medical forces for contingency medical support. As the air component surgeon to United States Special Operations Command (USSOCOM), the AFSOC/SG plans all Air Force medical support for AFSOC missions and serves as the principal Air Force medical service advisor to USSOCOM. AFSOC/SG is the waiver authority for all Command level requirements listed in this document.

1.3.2. Special Operations Wing (SOW) Surgeon. The SOW/SG is the Functional Area Manager for all operational medical personnel when present and provides advice and guidance to the SOW commander. Other responsibilities include:

1.3.2.1. Ensuring compliance with established medical policies in this instruction and other Air Force guidance.

1.3.2.2. Providing functional area review of medical unit reports and requests to higher headquarters.

1.3.2.3. Providing career mentoring to junior medical personnel and oversight of evaluation, awards, and decorations for assigned medical personnel.

1.3.2.4. Acting as liaison and consultant to AFSOC/SG for operational medical personnel issues.

1.3.2.5. Coordinating with line leadership for appropriate employment of operational medical personnel.

1.3.2.6. Establishing and maintaining relationship with host Chief of Aerospace Medicine (SGP) and Flight and Operational Medicine Clinic (FOMC) at the supporting MTF.

1.3.2.7. Ensuring operational medical personnel maintain their clinical skills and currencies by integrating clinic time at the host MTF.

1.3.3. Special Tactics Group Surgeon (STG/SG). The STG/SG coordinates medical training and oversees appropriate medical equipment for flight surgeons (FS), medics, and PJs assigned to Special Tactics Group (STG) and the Special Operations Surgical Teams (SOST). The STG/SG plans medical support for special tactics missions and selection of special tactics candidates. The STG/SG fosters a good relationship with host SGP and FOMC to ensure STG medics can maintain their clinical skills and currencies by clinically and administratively supporting home station and deployed MTFs. The Surgeon should also maintain a good relationship with host Base Operational Medicine Clinic (BOMC), as they accomplish fly-periodic health assessments (PHA), deployments, Aerospace Information Management System (ASIMS) tasks, and upgrade training for STG members. General oversight remains with the SOW/SG for SOWs without an STG/SG, while PJ medical oversight is described in **para 4.12**.

1.3.4. Medical Group Commander (MDG/CC). MDG/CC is responsible for oversight of all medical care on the installation. MDG/CC is the waiver authority for clinical currencies as established in this publication. This authority cannot be delegated.

1.3.5. Medical Group Chief of Aerospace Medicine (MDG/SGP). MDG/SGPs will oversee all activities related to flight and operational medicine (OpMed) in their MDG and will function as liaison between the MDG and all AFSOC operational medical personnel to include OCS. The MDG/SGP fosters good relationships with line medical assets and ensures OCS personnel can integrate with the host MDG.

1.3.6. SOW Medical Planner. SOW medical planners are Medical Service Corps (MSC) officers serving as the wing medical planner and deputy to the SOW Surgeon. The medical planner assists the Surgeon in executing all duties listed in 1.3.2 and serves as the wing medical administrator ensuring management and oversight of War Reserve Materiel (WRM) projects, budget forecast and execution, training programs, readiness reporting, and currency of unit manning documents. Wing medical planners will synchronize efforts with the Theater Air Operations Squadron. Additionally, medical planners are integral to planning medical support for deliberate and contingency missions. Medical planners will participate in all phases of operational exercise planning and execution.

Chapter 2

ADMINISTRATION OF MEDICAL ACTIVITIES

2.1. Command and Control (C2). For administrative and operational C2 in deployed settings, operational medical personnel shall follow C2 regulations per AFSOC, Director of Operations (AFSOC/A3) guidance. For in garrison chain of command, the flying or operational Squadron commander (SQ/CC) retains administrative control, operational control, and punitive authority in accordance with the Uniform Code of Military Justice over operational medical personnel. Rating chain will be through the flying or operational squadron. **(T-3)**

2.2. Privileging Documentation. All healthcare providers (HCPs) will ensure updated medical credentials in Joint Centralized Credentials Quality Assurance System available for rapid transfer to their deployment folders. Likewise, independent duty medical technicians (IDMTs) will maintain a copy of their current initial and sustainment training documentation in MyLearning. All IDMTs will hand carry these documents while deployed. If deployed to a location with medical facilities, HCPs and IDMTs will provide the medical facility commander with a copy of these documents. While deployed under USSOCOM orders, AFSOC operational medics assigned to line units remain within the special operations chain of command. However, AFSOC medical personnel will comply with co-located medical facility credentialing/privileging processes. The senior AFSOC physician at the deployed location will review Inter-facility Credentials Transfer Briefs and IDMT certification documents.

2.2.1. Non-privileged OCS assets. OCS assets discussed in this document refer only to the privileged providers.

2.2.2. The work provided by the nonmedical assets is overseen by the line squadron chain of command.

2.3. Quality Assurance (QA). The senior deployed AFSOC physician, or senior provider in absence of a physician, is responsible for medical quality assurance during deployments. Quality assurance will include peer review as described by the credentialing facility, both at home station and deployed. IDMT quality assurance will be completed IAW AFI 44-103, *The Air Force Independent Duty Medical Technician Program*.

2.3.1. After return from deployment, a copy of all chart review documentation (**Attachment 3**), Deployed Quality Assurance (QA) Oversight Documentation Sample), will be forwarded to the host MTF for continued credentialing and privileging actions.

2.3.2. All AFSOC credentialed and/or privileged healthcare providers, to include Preservation of the Family and Force (POTFF), True North (TN), Integrated Resiliency Optimization Network (IRON), Operational Support Teams, and Airmen Resiliency Teams (ART) will participate in peer review and, when necessary, standard of care reviews under the direction of the credentialing MTF.

2.4. Reporting. Operational medical readiness reporting as it relates to deployments and exercise participation will be submitted via After Action Report (AAR) or operational medical personnel Executive Activity Clinical Currency Report. Units are required to report resource readiness according to AFI 41-106, *Medical Readiness Program* Chapter 6.

2.4.1. Send reports to the AFSOC Chief of Medical Readiness Division (AFSOC/SGX) organizational email boxes at the appropriate level of classification: NIPR: **afsoc.sgx@us.af.mil**, SIPR: **afsoc.sgx@socom.smil.mil**. Time-sensitive reporting should always include positive personal contact by telephone.

2.4.2. IAW AFI 10-1302, *Air Force Lessons Learned Program*, personnel participating in contingencies, exercises, and AFSOC rotational deployments outside of continuous Air and Space Expeditionary Force (AEF) cycles will submit an AAR to Headquarters (HQ) AFSOC NLT 30 days, 60 days for ARC units, after the end of the event unless otherwise directed.

2.4.2.1. AARs should be submitted IAW theater, parent unit, or other local guidance, with courtesy notification to AFSOC/SGX. When no such guidance exists, AARs should be submitted directly to AFSOC/SGX. **Attachment 6** is the approved template.

2.4.2.2. Appropriate data repositories for AARs include, but are not limited to, the Joint Lessons Learned Information System, **https://jts.health.mil/index.cfm/documents/forms_after_action**, for all types of events and observations and the Joint Trauma System Trauma Registry. All trauma casualties treated/evacuated while deployed will have patient documentation/AAR submitted to Joint Trauma System (JTS) and Theater Special Operations Center (TSOC). Submissions for medical/training patients are highly encouraged. SIPRNet: **https://www.jllis.smil.mil** and NIPRNet: **https://www.jllis.mil**. Local guidance may mandate additional systems or processes.

2.4.2.3. In addition to the above mandatory reporting guidance, units and personnel should submit AARs whenever they make a significant observation that identifies a best practice or potential for poor outcome.

2.4.3. Executive Activity and Clinical Currency Quarterly Report (**Attachment 4**). All AFSOC operational medical personnel will complete quarterly (Jan - Mar, Apr - June, July - Sep, and Oct - Dec) executive summaries describing unit deployments and activities at an unclassified level and complete the Clinical Currency Report spreadsheet supplied by AFSOC/SGP. Requirements for the Clinical Currency Reports are outlined in the Training Tables available at: **https://usaf.dps.mil/sites/AFSOC-SG/SGX/SitePages/Home.aspx**. Reports are due NLT 15 days following the last day of the quarter to AFSOC/SGP and AFSOC/SGX, via NIPR, to **AFSOC.SGX@hurlburt.af.mil**. AFSOC/SGX will forward to other directorates and AFSOC/SG divisions as necessary.

2.4.4. Medical Logistics. Ensure training is completed as outlined in the AFSOCI 48-1010 Supplement, *Medical Training Tables*. Training Reports are due NLT 15 days following the last day of the quarter to AFSOC/SGX, via NIPR, to **AFSOC.SGX@hurlburt.af.mil**. All Geographically Separated Units will forward WRM Inventory Summary Letters upon completion of biennial WRM inventories, per AFMAN 41-209, *Medical Logistics Support* to the AFSOC Superintendent of Command Medical Logistics and The Deputy of the Medical Readiness Division (SGXL).

2.5. Operational Clinical Services (OCS). All AFSOC OCS will adhere to guidance found in AFMAN 48-149, *Flight and Operational Medicine Program (FOMP)*. OCS provide office-based medical and embedded mental health care, as well as spiritual and mental health resiliency support within designated squadrons, groups, and wings. OCS facilitates care by seeing Air Commandos at established clinics outside of the MTF. The advantages of this care model include improved access to care, enhanced understanding by the providers of the unique nature of the Special Operations Forces (SOF) mission and operations tempo, and an improved provider--Air Commando relationship, resulting in enhanced trust. OCS are medically staffed areas in line units where "pre-habilitation" and early medical intervention are provided for active duty, ANG and Reserve personnel on active orders only. Although not a component of the MTF, OCS will receive support and administrative oversight (e.g. credentialing, privileging, Military Health System Genesis (MHS-G) access and training, Health Insurance Portability and Accountability Act (HIPAA) training from the local MTF or other appropriate credentialing body per Department of Defense Instruction (DoDI) 6025.13, *Medical Quality Assurance*.

2.6. Medical Information Management. All patient encounters will be documented. All operational medical units must deploy with and use the laptop-based Theater Medical Information Program (TMIP) to document electronic healthcare records (EHR) with the Armed Forces Health Longitudinal Technology Application-Theater (AHLTA-T) application and/or a Battlefield Assisted Trauma Distributed Observation Kit (BATDOK) enabled End User Device (EUD). Failure to appropriately document patient encounters both in garrison and deployed constitutes a serious breach in standard of care.

2.6.1. Patient encounters completed in AHLTA-T and/or BATDOK applications will be uploaded to the Theater Medical Data Store (TMDS) as long as it remains operational or Operational Medical Data Service (OMDS) when it comes online from the deployed location. AHLTA-T utilizes a unique store and forward capability that allows data to be stored in a local database until connectivity to the Internet is restored, at which time data is automatically routed to TMDS. If no connectivity is available, the laptop may be connected at the first area of available communications. If users are unsuccessful, they can contact Air Force Operational Medicine Information Systems (AFOMIS) Reachback Support:

2.6.1.1. 24/7 AFOMIS Reachback Support DSN #: 312-596-5771 Option 1,1,3

2.6.1.2. 24/7 AFOMIS Reachback Support Commercial #: 334-416-5771 Option 1,1,3

2.6.1.3. 24/7 Support Commercial #: 210-630-7242

2.6.1.4. 24/7 AFOMIS Reachback Support Email: **fas.team1@us.af.mil**

2.6.1.5. AFOMIS Contact Email: **usaf.afomis-training@health.mil**

2.6.1.6. BATDOK also utilizes a unique store and forward capability that allows data to be within the EUD until connectivity to the internet or comms is restored and the encounter is deliberately transferred. BATDOK patient encounters can be transferred electronically multiple ways, but it is not automatic at this time.

2.6.2. Theater Medical Information Program-Joint (TMIP-J) and Joint Operational Medicine Information Systems (JOMIS) Modernized Health Care Delivery (HCD) Products Training. The Air Force Operational Medicine Information Systems (AFOMIS) team provides all TMIP-J and JOMIS Modernized HCD products training to the Air Force for all medical roles of care to include BATDOK, AHLTA-T, Theater Composite Health Care System (TC2), Medical Common Operating Picture (MedCOP), TMDS, OMDS, Operational Medicine Care Delivery Platform (OpMed CDP), Theater Blood-Mobile (TBLD-M), and MHS Genesis Theater (MHSG-T). AFOMIS provides trainers and equips them with same or equivalent OpMed training kits. There is no cost to AFSOC or individual units for training and support. AFOMIS provides designated contractors specific to AFSOC to provide subject matter expertise/advice, training, and exercise support where Operational Medicine Information Technology (IT) is utilized/required. Currently, these contractors are embedded with Special Operations Center for Medical Integration and Development (SOCMID) courses as well as the Special Operations Forces Medical Skills Development (SOFMSD) to provide OpMed IT training. However, they also travel to individual units to train and will accommodate unique/specific requests such as assistance with integrating OpMed IT into Point of Injury (POI), Prolonged Casualty Care (PCC), Damage Control Resuscitation/Damage Control Surgery (DCR/DCS), mass casualty (MASCAL), Disease Non-Battle Injury (DNBI), multimodal patient movement (MMPM), and Humanitarian and Disaster Relief (HADR) scenarios. Further, AFOMIS will be flexible in scheduling training/support time requirements.

2.6.3. All operational medical units will ensure UTC deployable computers and EUDs are functional before deploying. Any shortfalls or problems should be forwarded to local AFOMIS contractors (see 2.6.2 for contact), AFSOC/SGX and/or AFSOC/SGO for technical assistance.

2.7. Infection Control (IC) Responsibilities. IC is managed IAW AFI 44-108, *Infection Prevention and Control Program*, AFMAN 48-105, *Public Health Surveillance*, and Defense Health Agency Procedures Manual (DHA-PM 6025.13 Vol 2, *Patient Safety*).

2.7.1. In the deployed setting, the lead medic will ensure an active and current IC Program at all care locations and report any discrepancies in accordance with command policy.

2.7.2. Report IC discrepancies and inconsistencies to the flight commander, local MDG IC Function Chairperson, MDG Chief of Medical Staff (SGH), and group or wing SG.

2.8. Medication Dispensing Devices (MDDs). OCSs and POTFF clinics outside of the MTFs will not use MDDs for storage or dispensing of controlled substances.

2.9. Ancillary Services. When desired lab or radiology services are driven by care rendered in the OCSs, the local MTF will provide if available at the facility.

2.10. Self-Inspections. Self-inspection will be completed in Management Internal Control Toolset (MICT) following AF, AFMEDCOM, and AF Inspection Agency (AFIA) guidance. OCSs will undertake self-inspections at the frequency and in a manner as directed by AFIA guidance and instructions. At a minimum, two self-inspections are required annually. Line medical leaders should perform a self-inspection within 60 days of assuming the leadership role or sooner if local SOW policy dictates. The self-inspection program is one element of the quality program but does not cover all guidance applicable to the provision of medical care. All guidance applicable to the provision of medical care by Air Force providers also applies to the care provided by operational medical personnel assets.

2.11. Funding for Supplies. Funding for medical supplies for the OCS may be provided using Defense Health Program (DHP) funds as permitted by Air Force policy or through line funding. Non-disposable OCS medical supplies and equipment are funded using Line sources, and day-to-day disposable medical supplies are funded through DHP sources. OCS medical supplies will be ordered through the MTF supply chain and the MTF will assist with ordering and procuring medical equipment and supplies as needed.

2.11.1. WRM funding is for deployed medical care. WRM supplies will be used in compliance with all guidance for WRM supplies and will not be used for OCS patients for in-garrison care. Expendable stock, such as medications, may be rotated out of WRM and used before its expiration date. WRM should be maintained in accordance with AFMAN 41-209 Chapter 8.

2.11.2. **Operational Assemblages.** Initial outfitting of the operational assemblages is funded by AFSOC/SGX. Authority, direction, and control of the assemblages is aligned to the unit commander. The unit is responsible for maintaining and purchasing of sustainment items, and resupply.

2.11.3. Qualified aviation personnel filling API-5 or Operational Support Flying billets are authorized to use their organization operations and maintenance (O&M) type funds as directed by their commanders or other competent authority IAW DAFMAN 65-605V1, *Budget Guidance and Technical Procedures* paragraph 11.13.7 to purchase flight specific uniforms and equipment. This applies to operational and MTF assigned aviators.

Chapter 3

FORCE HEALTH AND GROUNDING MANAGEMENT

3.1. Objectives. To ensure protection and promotion of Airmen's health in garrison and when deployed. Familiarity of aircrew physical standards, deployment health requirements, and health surveillance are integral to force health and grounding management.

3.2. Aircrew Physical Standards. When making aeromedical dispositions, AFSOC aeromedical providers must refer to DAFMAN 48-123, *Medical Examinations and Standards*, the Medical Standards Directory, AF and AFSOC policy letters, and the USAF Aerospace Medicine Waiver Guide published by the Aeromedical Consultation Service on the Air Force Medical Service (AFMS) Knowledge Exchange.

3.2.1. Disqualification Authority. Refer to DAFMAN 48-123 for certification/disqualification authority. Local disqualification is not authorized.

3.2.2. Aeromedical Consultation Service (ACS). All ACS evaluation requests must be submitted through the Aeromedical Information Management Waiver Tracking System to the AFSOC Chief of Flight Medicine (SGPA) for review and subsequent forwarding to the ACS.

3.3. Diving Medical Standards. Special Warfare Airmen (SWA) require medical clearance to perform diver operations IAW DAFMAN 48-123, *Medical Examinations and Standards*, and AFI 10-3504, *Dive Program*. The SWA annual special duty ("flight") physical clears ST operators for operational diving and for any formal Air Force dive course of instruction. A current DD Form 2992, *Medical Recommendation for Flying or Special Operational Duty* provides proof of medical clearance for diving operations unless annotated otherwise in Box 13 of the DD Form 2992. SWA will follow the appropriate sister service medical and physical standards when attending formal non-AF dive courses. Navy dive courses (e.g. Dive Supervisor) require a dive physical IAW Navy MANMED Chapter 15. Army dive courses require a physical IAW AR 40-501, *Standards of Medical Fitness*. These can be accomplished by the unit flight surgeon and certified by the respective dive school Underwater Medical Officer/Dive Medical Officer (UMO/DMO).

3.4. Aeromedical and Special Duty Disposition. All return to flight status or return to special operations duty status (e.g. control, jump, dive) require grounding management dispositions by a licensed and credentialed Flight Surgeon (FS), Aeromedical Physician Assistant (APA), or Aeromedical Nurse Practitioner (ANP). Duties Not to Include Flying (DNIF) and Duties Not Including Controlling (DNIC), jumping, or diving may be completed by any clinical provider or IDMT.

3.4.1. Adhere to AFMAN 48-149 on the requirements for the training and experience required for APAs and ANPs to return a flyer or special warfare airman (SWA) to duty status.

3.4.1.1. IDMTs must contact a US military flight surgeon for appropriate aeromedical disposition.

3.4.1.2. This contact must be documented in the aviator's or special warfare airmen's medical record and subsequently countersigned by the consulted aeromedical provider upon return from deployment.

3.4.2. AF Form 1041, *Medical Recommendation for Flying or Special Operational Duty Log Review*. A representative from the AFSOC operational medical unit shall attend and participate in the weekly Flight and Operational Medicine Working Group at the host MTF. Operational aeromedical providers shall actively monitor and manage the aeromedical status of all DD Form 2992 holders in their assigned units.

3.5. Airmen Medical Readiness Optimization (AMRO). AFSOC operational medical personnel will participate in local AMRO Boards per DAFMAN 48-108, *Physical Evaluation Board Liaison Officer (PEBLO) Functions: Pre-disability Evaluation System (DES) and Medical Evaluation Board (MEB) Processing*. The AMRO Board reviews service members with duty limiting conditions that affect mobility, retention, or long-term physical fitness. A provider who is familiar with the service member's case will be present to give accurate representation.

3.6. Use of Fatigue Countermeasure Medications. Non-pharmacologic fatigue countermeasures (e.g., scheduling, aircrew augmentation, etc.) are the preferred methods of combating fatigue and making operational risk management (ORM) decisions. Fatigue countermeasure medications are an additional mitigation strategy available in consultation with a senior flight surgeon when a fatigue risk remains despite a commander's best efforts to use non-pharmacologic countermeasures. These medications are not a solution for manning shortages, suboptimal operational planning, support of a lifestyle of poor sleep habits, or inappropriate prioritization of work and play activities. Because of their addictive properties and potential for abuse, fatigue countermeasure medication used by aircrew and SWA is controlled and requires coordination and oversight by both flight medicine and line leadership.

3.6.1. Approval authorities.

3.6.1.1. No-Go pills. AFSOC/SG delegates approval authority for operational use of No-Go pills to the FS, APA, and ANPs at the operational unit level. Authority delegation is contingent upon full compliance with all program and documentation requirements. The senior flight surgeon at the unit level is responsible for implementation of this policy.

3.6.1.2. Go pills.

3.6.1.2.1. AFSOC/A3 is the approval authority for operational use of Go Pills for aircrew members.

3.6.1.2.2. Special Tactics Group (STG)/CC is the approval authority for operational use of Go Pills for SWA members.

3.6.2. Planning. The Wing representative and MTF/SGP will develop a fatigue countermeasure plan for mission types in advance of deployment, based on the unit's alert or short notice requirements, and provide a copy to the AFSOC/SGP. If the unit is a tenant unit on another Major Command's (MAJCOM) installation, the AFSOC/SGP will be consulted. The fatigue countermeasure plan will be reviewed annually.

3.6.3. Approved fatigue countermeasure medications. AFMEDCOM publishes the list of authorized stimulants ("Go Pills") and sedatives ("No-Go Pills") in the *Official Air Force Aerospace Medicine Approved Medications List.* Aeromedical clinicians must supervise the use of any fatigue countermeasure medications and exercise careful judgment before dispensing.

3.6.4. Considerations for use. The decision to prescribe and use fatigue countermeasure medications in a specific situation is one between the Airmen and their flight surgeon. It is a voluntary decision based on anticipated levels of fatigue and alertness while participating in a mission or situation where fatigue countermeasure medications are already authorized.

3.6.5. No-Go Pills. No-Go Pills are a useful adjunct for adjusting an aircrew's or SWA's circadian rhythm to coincide with crew duty day.

3.6.5.1. This policy for use of No-Go pills also applies to all AFSOC operational units to include: MTF FOMC, aviation, special tactics, remote piloted aircraft (RPA), and Special Operations Theater Air Operational Support (SOTAOS) units.

3.6.5.2. Aircrew and Special Operations Duty personnel are verbally DNIF for 12 hours after taking Temazepam (Restoril®), 6 hours after taking Zolpidem (Ambien®), or 4 hours after taking Zaleplon (Sonata®) IAW *Official Air Force Aerospace Medicine Approved Medication*.

3.6.5.3. No-Go pill use of Temazepam and Zolpidem is limited to a maximum duration of 7 consecutive days and no more than 20 days in a 60-day period. Zaleplon may be used for up to 10 consecutive days and no more than 28 days in a 60-day period.

3.6.5.4. No-Go pills are authorized during transition from deployment or redeployment settings to synchronize circadian rhythm for deploying or redeploying crews. Short term use of No-Go pills is also authorized to assist deployed crew members with fatigue and circadian rhythm problems.

3.6.6. Go Pills. Go Pills can improve alertness, vigilance, and ultimately safety for aircrew members during long missions or critical phases of flight, and for SWA during prolonged missions. They are used when a fatigue deficit exists during a mission despite the best efforts of leadership and aircrew or SWA to use non-pharmacologic fatigue countermeasures.

3.6.6.1. In general, Go Pills are potentially appropriate for missions that disrupt an aircrew member's or SWA's normal sleep-wake cycle, including but not limited to: multi-crewed aircraft missions greater than 12 hours or when crew duty day is expected to exceed 16 hours; fighter/single-pilot or helicopter missions greater than 8 hours duration; alert missions; combat missions for crews shifting from day to night with only 12 hour notice; and Special Warfare (SW) missions, when SWA may not return to base for a prolonged time, and do not have the benefit of fixed crew rest.

3.6.6.2. Go Pill use may be authorized for missions when there is no disruption to an aircrew member's or SWA's normal sleep-wake cycle, but use would otherwise be beneficial to the mission. The base SGP (or a flight surgeon supporting an AFSPECWAR unit) will coordinate with AFSOC/SGP for these circumstances. (For tenant units contact their MAJCOM SGP.)

3.6.6.3. For aircrew members and SWA, Go Pill use will be confined to approved medications in the most current version of the *Official Air Force Aerospace Medicine Approved Medications*. None are authorized for routine clinical use in flyers or special duty personnel.

3.6.6.4. Remotely Piloted Aircraft aircrew members are not authorized to use Go Pills.

3.6.6.5. Go Pill use is not authorized for parachute or diving operations.

3.6.7. Oversight. The use of fatigue countermeasure medications will be reviewed at Aeromedical Council and with the SOW/CC's delegate or STG/CC representative at least every six months. A copy of the usage report will be forwarded to the AFSOC/SGP. For tenant units, also send copy to their MAJCOM SGP. The review should identify groups or individuals that appear to be under- or overusing medications and should also focus not only on medical but on the line handling of tasking and early interventions that can mitigate the need for pharmacological countermeasures (e.g., ensure schedulers are planning efficiently). Reports should include aircraft type and number of sorties (number of missions for SWA) where a fatigue countermeasure medication was prescribed, quantity and strength prescribed, quantity used, efficacy, any untoward side effects, as well as identifying best practices. A sample format can be found on the "Go Pills" link at the Knowledge Exchange (Kx).

3.6.8. Ground Testing. All initial ground testing will be accomplished when new aircrew are assigned to their initial technical school, during the first few weeks of academics—i.e., when they are not actively flying. SWA will ground test at the earliest opportunity. All aircrew and SWA ground testing fatigue countermeasure medications must receive counseling regarding off label use of Go Pills as an operational countermeasure and sign an informed consent prior to use per DoDI 6200.02, *Application of Food and Drug Administration (FDA) Rules to Department of Defense Force Health Protection Programs*, and AFMAN 48-149, *Flight and Operational Medicine Program*. Informed consent forms can be found on the AFMS Knowledge Exchange: **https://kx.health.mil/kj/kx4/FlightMedicine/Pages/CurentGoPillFormsNov2011.aspx**

3.6.9. Transport. Aeromedical clinicians and other healthcare clinicians transporting controlled substances on temporary duty or deployment.

3.6.9.1. Aeromedical clinicians or technicians may transport controlled substances issued by the MTF pharmacy or medical logistics specialist in the context of operational support to line units IAW AFMAN 48-149. Clinicians or technicians should only transport controlled substances on temporary duty or deployment when pharmacist or pharmacy technician is not available.

3.6.9.2. When transporting controlled substances, the clinician will:

3.6.9.2.1. Be issued an AF Form 579, *Controlled Substances Register*, by the local MTF pharmacy, Medical Service Corps, or logistics technician.

3.6.9.2.2. Possess a memo signed by the MTF SGP or SOW/SG authorizing transport of controlled substances. This memo should address the rationale for transporting controlled substances; validation that controlled substances were issued and documented on the AF Form 579; any limitation on dispensing controlled substances; and a POC.

3.6.9.3. Unused medications may be collected and disposed of IAW AFI 44-102, *Medical Care Management.*

3.7. Performance Enhancement (PE).

3.7.1. PE is the act or process of increasing one's level of achievement at a task or activity by building physical and mental capacity or endurance for the activity. PE requirements set forth in this instruction do not apply to credentialed medical providers using FDA approved devices or therapeutics. These providers will follow established clinical practice guidelines and clinical indications of devices or biologics.

3.7.1.1. PE Devices: PE devices may range from commercial monitors, trackers, force plates, and apps to biotechnology devices and physical and cognitive stimulation devices (e.g., biofeedback monitors or wearable devices that monitor or capture sleep cycles or heart rate variability). Such PE devices differ from therapeutic treatments, which are used to treat medical diagnoses, symptoms, or other identified deficiencies. Many are not regulated by FDA guidelines or oversight thus application and results can have varying levels of efficacy and risk.

3.7.1.2. For all AFSOC units, purchasing and fielding of a Performance Enhancing (PE) device must be approved by the unit commander with full knowledge of the potential risks they are accepting. To enable this, prior to purchasing a PE device units must vet their requests through HQ AFSOC/A1Z and HQ AFSOC/SGR for an Effectiveness and Risk Determination letter and receive written unit commander indorsement on said letter accepting the risks inherent in purchase of the device. AFSOC/SGR maintains a database of approved devices and will send a link to that database upon placing a request to this org box: **AFSOCSG.SGR.distribution@us.af.mil**.

3.7.1.2.1. A1Z and SGR shall initiate review of all non-biased data and assessments on PE initiatives from an expert source (e.g., 711 Human Performance Wing (HPW)), which will consider use of that system for the intended operational purpose(s) in the intended operational environment(s). The letter will cover product effectiveness for intended use (A1Z), potential risk to mission (A1Z), potential risk to human health (SGR), and a human subject research pre-determination (AFSOC research gatekeeper). In addition, HQ AFSOC/SGP will evaluate any PE device used by aircrew or special operational duty personnel. AFSOC/SGP will be the final approval authority for use of such PE devices intended for aircrew and/or special operational duty personnel.

3.7.1.2.2. Units will send all requests for an Effectiveness and Risk Determination letter to **AFSOCA1.A1Z.AllPersonnel@us.af.mil** and **AFSOCSG.SGR.distribution@us.af.mil**. The following information must be included in any such request:

3.7.1.2.2.1. Product information (e.g., make/model, specification sheet, website link, etc.)

3.7.1.2.2.2. Perceived benefit of purchasing product

3.7.1.2.2.3. Intended use of product

3.7.1.2.2.4. Who will use product (e.g., medical provider, non-medical individuals, etc.)

3.7.1.2.2.5. The potential “patient” population (e.g., active-duty members of unit)

3.7.1.2.2.6. Signature block and organizational email address for unit commander approving purchase

3.7.1.3. If the device/technology is part of a research study/Institutional Review Board (IRB) controlled, it will only be used as a part of the approved study and will not be distributed or used by AFSOC personnel in any other manner. Any units wishing to use these devices outside of approved research protocols must follow Effectiveness and Risk Determination steps detailed above.

3.7.2. Dietary Supplement Usage.

3.7.2.1. Dietary Supplement Usage is per USSOCOM Policy Memorandum 24-04, *U.S. Special Operations Command Policy for Commercial Off the Shelf Nutrition Products*, 12 February 2024 and DoDI 6130.06, *Use of Dietary Supplements in the DoD*.

3.7.2.1.1. All AFSOC personnel on flight and special operations duty status must be granted prior approval by an aeromedical provider for use of any dietary supplement. Per DAFMAN 48-123, personnel are "down" for any unauthorized use.

3.7.2.1.2. Specific supplement and rationale for use must be documented in the aircrew or operator's medical record.

3.7.2.2. While dietary supplements may be approved for use, the use of O&M funds to obtain approved products is not authorized.

3.7.2.2.1. Operation Supplement Safety (OPSS) a resource from the Consortium for Health and Military Performance (CHAMP), provides an expansive list of substances that have at one time appeared, or currently appear, as ingredients in products labeled as dietary supplements that FDA or the U.S. Armed Services have disallowed.

3.7.2.2.2. Aeromedical providers may utilize the OPSS as a reference to make informed when advising AFSOC personnel on use of approved dietary supplements.

3.8. Blood Borne Pathogen (BBP) Exposure Risk and Control Plan.

3.8.1. AFSOC medical operational units will establish an exposure control plan for care and evaluation of AFSOC assets and those that fall under their medical responsibility in the field who may have exposure to blood or body fluids in an austere environment. The protocol should not replace the standing post exposure protocol at the in-garrison MTF for the active duty member upon their return from the austere location.

3.8.2. In accordance with *Updated US Public Health Service Guidelines for the Management of Occupational Exposures to Human Immunodeficiency Virus (HIV) and Recommendations for Post-Exposure Prophylaxis* (PEP), medical providers who are conducting high risk training or have a high expectation of exposure to HIV must deploy with PEP. If diagnostic laboratory resources are not available to test either a source patient or medical provider prior to or following a BBP exposure, the exposure must be documented. The medical provider will complete BBP exposure testing and treatment at the next available opportunity.

3.8.2.1. Local plans should include education of deploying forces with pre-deployment briefings on HIV, Hepatitis B Virus, and Hepatitis C Virus risks in theater. Education of line members should emphasize post exposure cleanup after Tactical Combat Casualty Care (TCCC), the use of personal protective equipment (PPE), and the definition of exposure.

3.8.2.1.1. Significant exposures are considered blood or body fluids on mucous membranes or non-intact skin and/or blood or body fluids injected via a needle stick from a hollow or solid needle, or other sharp objects.

3.8.2.1.2. Non-significant exposures are considered blood or body fluids on intact skin, casual contact with an HIV-infected individual, or exposure to an animal's blood or body fluids.

3.8.2.2. Education of medical forces should emphasize universal precautions and use of PPE, especially in the use of sharp instruments in a low light combat environment which places SOF medics at increased risk.

Chapter 4

AFSOC OPERATIONAL MEDICINE

4.1. AFSOC Operational Medical Personnel Responsibilities. All AFSOC operational medical personnel will work toward the successful execution of the Aerospace Medicine Enterprise as defined in AFI 48-101.

4.1.1. AFSOC operational medical personnel deliver healthcare, both in-garrison and while deployed. Flight surgeons, SOST members, APAs, ANPs, psychologists, licensed clinical social workers, nurses, IDMTs, physical therapists, health service management personnel, and medical technicians designated as operational medical personnel to AFSOC units have unique roles and responsibilities. When in-garrison, and when not participating in mandatory squadron functions, training, or exercises, operational medical personnel are required to maintain clinical proficiency, meet medical training and administrative requirements, and maintain clinical credentials and privileges. Meeting and sustaining the requirements contained herein qualifies AFSOC's operational medical personnel for deployment. Unit commanders shall employ their medical assets meaningfully. Therefore, operational medical personnel will not be assigned duties that normally would be performed by non-medical personnel (e.g., readiness manager).

4.1.2. AFSOC's operational medical personnel must be prepared for operational deployment. Clinical competency and proficiency are the most important and critical qualifications AFSOC operational medics bring to the fight. These skills are extraordinarily perishable. All operational medical personnel, without exception, must meet or exceed their duty clinical requirements. In order to maintain these perishable qualifications, the primary emphasis of in-garrison activities will be prioritized to the delivery of safe and effective healthcare in clinic and hospital settings. AFSOC operational medical personnel must complete the clinical exposure requirements listed in the AFSOC Medical Training Tables, as well as AFSC specific requirements.

4.1.2.1. Rules of Engagement while Deployed. AFSOC medical personnel must function within the parameters of the established mission set. No additional engagement activities will be conducted without appropriate level of approval. The chain of command will decide which medical support requests will be channeled to the embassy and/or Department of State entities in the Area of Responsibility (AOR).

4.1.2.2. When in-garrison, AFSOC operational medical personnel will maximize their clinical time in the affiliated hospitals, MTF, or OCS settings while performing duties directly related to the hands-on delivery of healthcare services. MTF and clinic settings include, but are not limited to: Flight Medicine, BOMC, Family Practice, Emergency Department, Pediatrics, Radiology, and specialty clinics. Duty schedules will be developed and published, with best available information, for operational medical personnel and will be shared and coordinated with the host SGP at least 30 days prior to clinical duties. Host MTF will ensure patients, examination rooms, and operating rooms are scheduled for operational medical personnel.

4.1.2.3. Operational medical personnel may practice their full scope of credentialed privileges as long as the MTF is able to provide full administrative support, and medical care is performed at the MTF.

4.1.2.4. Each OCS provider's patient care time will be reported by SOW/A3M or designee to the AFSOC/SGP quarterly within 15 days after the last day of the quarter, i.e. 15th of January, April, July, and October. Clinic time will be reported as the number of half-days of patient care performed by the provider for the reported quarter. The number of patients each provider sees and treats as the treating provider will be included in this report, i.e. merely signing off as a supervising provider or determining aeromedical disposition does not qualify for patient count purposes.

4.1.3. All AFSOC operational medical personnel will comply with USAF, Defense Health Agency (DHA), AFMS, AFMEDCOM, and host MTF requirements that govern the delivery of healthcare. To remain in good standing with the host MTF, all healthcare providers, including those assigned directly to line units, must complete all training and credentialing requirements. Additionally, operational medics who are considered professional staff must attend host MTF professional staff meetings. Unit Commanders will ensure assigned operational medical personnel meet clinical exposure requirements. AFSOC medical personnel who fail to meet clinical competency and currency requirements may be subject to medical privilege abeyance and/or credentials action IAW AFI 44-119, *Medical Quality Operations* and DoDI 6025.13, *Medical Quality Assurance and Clinical Quality Management in the Military Health System*.

4.1.4. All AFSOC medical personnel will comply with AFI 44-102, *Medical Care Management*, when seeking or engaging in off-duty employment. Patients seen in off-duty employment settings will be counted toward clinical currency requirements at the discretion of AFSOC/SGP.

4.1.4.1. All line assigned healthcare personnel must first obtain the written permission of the line squadron commander or next highest authority as appropriate. This written permission must include information outlined in AFI 44-102 para 2.27.2.3. Non-MTF providers also require permission from the most senior medical officer in their chain of command per para 2.27.2.4.. This authority will often fall to the SOW/SG.

4.1.4.2. AFSOC medics on non-AFSOC bases must establish a written memorandum between the line unit and the MTF which will describe the process for completing the requirements for off-duty employment.

4.1.4.3. For line-assigned medics, the MTF/CC or the line CC may rescind permission for off-duty employment at any time.

4.1.5. When in-garrison, IDMTs will perform IDMT sustainment training/duties IAW AFI 44-103, *The Air Force Independent Duty Medical Technician Program,* Career Field Education and Training Plan (CFETP), and the applicable training tables within this document

4.1.6. AFSOC operational medical logistics personnel and MSC officers will ensure medical equipment and supplies are ready for immediate deployment. Additionally, the medical logistics point of contact (POC) for each operational medical element will coordinate with the host MTF medical logistics section to ensure proper calibration, maintenance, and repair of medical equipment.

4.1.7. AFSOC MSCs assigned or attached to operational medical elements will directly participate in mission planning as the primary medical planner. Operational medical personnel will participate in mission train-up and mission rehearsal as required.

4.1.8. The host MTF retains primary responsibility for medical readiness for all total force and eligible personnel. However, operational medical personnel will assist the host MTF regarding pre-deployment preparations and individual medical readiness requirements of AFSOC flyers to include fly PHAs and Deployment Readiness Health Assessment (DRHA) 1s on a basis of OCS availability.

4.1.9. For operational medical personnel not assigned to a SOW or an installation with an AFSOC MTF, operational medical personnel will review, track, and maintain Preventive Health Assessments and ASIMS mobility requirements. Furthermore, operational medical personnel shall assist in pre- and post- deployment medical screening interviews, and coordinate with the Alert Deployment Clearance Function and host MTF force health management regarding assigned AFSOC members. To ensure they have the most current unit personnel information, they will provide representation at MTF grounding management activities. AFSOC operational medical personnel will report Preventive Health Assessment and Individual Medical Readiness (IMR) data to appropriate AFSOC unit commander(s).

4.1.10. After completing the immunization backup technician course, OCS clinical medics will assist host MTF with the administration of immunizations to deploying AFSOC forces unless they are deploying themselves and are therefore unable to assist. They will take the lead in providing immunizations for their assigned units.

4.2. SOW/A3M, Medical Operations Division. Operational medical personnel require a unifying source of oversight to ensure standardized readiness and to advise commanders on optimal employment.

4.2.1. A3M will be composed of the SOW/SG team, which includes the flight surgeon, medical planner, and medical logistician.

4.2.2. In addition to fulfilling the responsibilities outlined in **paragraphs 1.3.2. and 1.3.6.,** A3M provides oversight of personnel management, training requirements, WRM assets, Status of Resources and Training System (SORTS), Medical Readiness Decision Support System (MRDSS), AEF Reporting Tool, Defense Readiness Reporting System (DRRS), and other reporting requirements.

4.3. Special Operations Forces Medical Element (SOFME). SOFMEs are deployable, operational medical units within the SOW. They provide base operational support, PCC, initial combat trauma stabilization, Advanced Trauma Life Support (ATLS), Advanced Cardiac Life Support/Advanced Life Support (ACLS/ALS), and MMPM for supported forces as mission circumstances and requirements dictate in austere locations.

4.3.1. The primary responsibility of SOFME personnel is full-spectrum aerospace medicine support to AFSOC aircrew members and special duty personnel, as described in AFMAN 48-149 chapter 3.

4.3.1.1. The assigned unit should be where the SOFME providers accomplish the bulk of mission essential task list (METL) requirements, and for whom they primarily provide full-spectrum aerospace medicine support.

4.3.1.2. Each flying unit should be able to contact their SOFME team when in garrison. The on-call MTF flight surgeon should not be the default response. Note: This is not applicable for ARC.

4.3.2. SOFME personnel perform the following functions as part of deployed operations:

4.3.2.1. Provide aeromedical clinical and operational medical support.

4.3.2.2. Provide initial trauma care and resuscitation for supported forces.

4.3.2.3. Provide PCC and MMPM for supported forces.

4.3.2.4. Provide base occupational and environmental assessments for supported forces.

4.3.2.5. Provide baseline and ongoing assessments and mitigation of potential environmental and occupational health hazards and water vulnerability assessments. SOFME personnel will document environmental and occupational health exposures, known or potential exposure to chemical, biological, radiological, and nuclear (CBRN) agents, or other health risk exposures.

4.3.2.6. Provide line commanders guidance on the use of pyridostigmine tablets, CBRN defensive auto-injectors, or other similar agents.

4.3.2.7. SOFMEs may be tasked to conduct Global Health Engagement (GHE) activities during Special Operations Task Group (SOTG) deployments, Joint Combined Exchange Training (JCET), TDYs, and other theater exercises. Refer to section 4.5.1 for additional guidance on GHE.

4.4. Special Operations Surgical Team (SOST). SOST personnel provide DCR/DCS, post-surgical critical care management, MMPM, and critical care transport at austere forward locations in permissive, semi- and non-permissive threat environments. SOST personnel fulfill GHE taskings to train partner nations on advanced resuscitation and surgical capabilities. The SOST includes a General Surgeon (45S3), Nurse Anesthetist (46M3), Critical Care Nurse (46N3), Emergency Physician (44E3A), Surgical Technician (4N1), and Cardiopulmonary technician (4H0). Suitable substitutions are Anesthesiologist (45A3) for Nurse Anesthetist (46Y3M) or Emergency Physician (44E3A) and Emergency Room Nurse (46N3J) for Critical Care Nurse (46N3E). SOST personnel provide full-spectrum SOF surgical support, post-surgical critical care management, MMPM, and critical care transport at austere forward locations to include the point of injury.

4.4.1. Deployment. SOSTs are configured to support enduring and alert missions. Because downrange surgical procedures may not be common, SOSTs should not be deployed for prolonged increments (i.e. 6 months). Waivers for longer deployments must be approved by AFSOC/SG.

4.4.1.1. SOST Individual Mobilization Augmentees (IMA) will deploy IAW Air Force Reserve Command (AFRC) directives and AFI 10-402, *Mobilization Planning*.

4.4.1.2. HQ Readiness Integration Office (RIO) Installation Personnel Readiness (IPR) will process all IMA deployment orders.

4.4.2. SOST will copy information related to daily Situation Report (SITREP) as required by AFSOC/A3 with the following information through SIPR channels to AFSOC/SGX**.**

4.4.2.1. Current location.

4.4.2.2. Number of patients treated and type: US, coalition, friendly, enemy combatant, civilian.

4.4.2.3. Number of surgeries/procedures accomplished.

4.4.2.4. Amplifying data deemed appropriate.

4.4.3. SOST personnel will meet all initial and mission qualification training requirements as well as currency training requirements as prescribed in the instruction, AFSOC Medical UTC Training and Currency Requirements.

4.5. Theater Air Operations Squadrons. Medical personnel assigned to the SOTAOS plan and execute GHE exchanges and activities to enable strategic objectives in their respective theater of alignment. As the medical component to the multidisciplinary Air Advisor team, their primary responsibility is to assess, train, advise and assist strategic foreign partners on health-related matters. The operational medics assigned to the SOTAOSs are trained as Air Advisors and International Health Specialists (IHS) and will obtain the corresponding IHS Special Experience Identifier. AFSC specific medical training requirements and criteria are outlined in Medical Training Tables located at: **https://usaf.dps.mil/sites/AFSOC-SG/SGX/SitePages/Home.aspx**.

4.5.1. Global Health Engagement. GHEs are any health-related interactions between the DOD and Partner Nations' armed forces or civilian authorities to build trust, share information, and achieve interoperability in support of U.S. national security policy and military strategy. While medics of the SOTAOS are primarily responsible for the strategy and coordination of GHE with the TSOCs of their respective theater, AFSOC medical personnel assigned to UTCs routinely conduct GHE with partners during training, exercises, and operational missions.

4.5.2. Any and all medical engagements with foreign partners will be planned and coordinated through the respective Wing A3M to include requesting additional support and resources for planned activities. All engagements will be conducted IAW DODI 2000.30 *Global Health Engagement Activities* and AFI 44-162 *International Health Specialist (IHS) Program and Global Health Engagement (GHE)*. Assessment, Monitoring and Evaluation (AM&E), storyboards and AARs will be accomplished for all GHE activities and submitted to the appropriate AFSOC SGX email. SIPR AFSOC SGX Org Box: **afsoc.sgx@socom.smil.mil**; NIPR AFSOC SGX Org Box: **afsoc.sgx@us.af.mil**.

4.6. Special Operations Psychologist (Ops Psych). Ops Psychs provide operational support to SOF missions and personnel. Although the provision of healthcare is not the primary support mission of Ops Psychs, there may be instances when it is appropriate for Ops Psychs to provide clinical support or triage in order to secure definitive care for a service member.

4.6.1. Ops Psychs advise commanders and SOF personnel on conventional and unconventional behavioral science principles affecting a variety of in-garrison and operational activities. These activities include, but are not limited to: assessment and selection of personnel, unit and individual performance optimization, stress inoculation, leadership coaching, oversight for Survival, Evasion, Resistance, and Escape (SERE) training and exercises, post-mishap consultation, combat stress control measures and return-to-duty support, reintegration of recovered personnel after isolation or capture, consultation to interrogation and detention operations, support for Influence operations and strategic communication, adversary profiling, and support to HUMINT collection activities. They can also provide human factors consultation to aircraft and incident mishap investigations and accident investigations if available, but the more appropriate/specialized consultant would be the aviation psychologist, as described below.

4.6.2. All Ops Psychs must maintain active credentials at a DOD-approved MTF and should sustain an appropriate level of operational and clinical currency to support their credentials as well as their primary duties and responsibilities as outlined above. When providing healthcare support within the MTF, all ops psychs will adhere to the requirements of the base MTF.

4.7. Aviation Psychologist. Aviation Psychologists are specially trained to perform duties in the domains of human performance, aviation assessment and selection, human factors engineering, simulation and training, and safety. They provide mission support capabilities in training, command consultation and coaching, performance enhancement, and specialized intervention services within aerospace medicine. Aviation Psychologists require continuous exposure and familiarization of crew and individual dynamics in the aircrew environment. In order to accomplish this, they are placed on non-interference aeronautical orders, flying a minimum of four hours per month.

4.7.1. Operational Unit-Level Responsibilities: When assigned to an operational flying unit, they perform aeromedical psychological fitness and suitability evaluations, consult on the mental health portion of aeromedical waivers, consult with flight medicine and mental health providers to advise on aircrew psychological treatment, participate in operational/training flying missions to assess psychological or human factors, stressors, and demands, and employ human factors research and statistical analysis. They are the preferred psychology human factors expert to support Air Force Safety Investigation Boards (SIBs).

4.7.2. Training Unit-Level Responsibilities: When assigned to a flying training unit, aviation psychologists provide education and training in aviation psychology, consult on fatigue mitigation behavioral management, deliver and advise on proactive intervention for stress reduction, oversee airsickness management programs, and supervise other prevention services.

4.7.3. MAJCOM Safety Responsibilities: When assigned to a Safety billet, they focus on human factors and aviation/occupational safety at the organizational level. They provide direct and indirect support to SIBs, conduct organizational safety assessments, participate in human factors research, and apply psychological and human factors principals to mishap prevention.

4.7.4. All aviation psychologists must maintain active credentials at a DOD-approved MTF and should sustain an appropriate level of operational and clinical currency to support their credentials as well as their primary duties and responsibilities as outlined above. When providing healthcare support within the MTF, all aviation psychologists will adhere to the requirements of the base MTF. Although the provision of healthcare is not the primary support mission of aviation psychologists, there may be instances when it is appropriate for them to provide clinical support or triage in order to secure definitive care for an aviator.

4.7.5. Aviation psychologists are authorized to participate in aircraft flights as observers on a noninterference basis. Host Aviation Resource Management (HARM) offices will publish aeronautical orders according to this instruction. Neither Aviation Incentive Pay (AvIP) entitlement nor Operational Flying Duty Accumulator (OFDA) credit are authorized.

4.8. Special Operations Aeromedical Physician Assistants (APA) and Special Operations Aeromedical Nurse Practitioners (ANP).

4.8.1. All AFSOC APA and ANP positions must be filled by APAs and ANPs with no less than 24 months of clinical PA or NP experience and must have attained and maintained appropriate credentials on their privilege list. AFSOC APA and ANPs assigned to OCS may support deployment missions and operations, although their main role is to provide primary care services and support, preventive healthcare services, and healthcare continuity for in-garrison AFSOC aircrews, and special duty personnel. APA and ANP assigned to an operational squadron, especially those on FFQEK and FFQE8 UTCs, will support deployment missions and operations.

4.8.2. When APAs and ANPs deploy, they function under MTF privileges and through physician consultation at all times either in person, by phone, or electronic means.

4.8.2.1. A senior flight surgeon must be identified in writing for each APA or ANP as physician collaborator, as well as an alternate. This information will be placed in Section 1 of each APA or ANP's credential file.

4.8.2.2. APAs and ANPs will not deploy to an area likely to require skills outside of credentialed scope of their medical practice.

4.8.3. APAs and ANPs may practice appropriate aeromedical dispositions if they meet the requirements and training criteria outlined in AFMAN 48-149 and AFSOC AFSC specific training requirements in the training table located at: **https://usaf.dps.mil/sites/AFSOC-SG/SGX/SitePages/Home.aspx**.

4.9. Special Operations Independent Duty Medical Technicians (SOIDMT).

4.9.1. All SOIDMTs within Special Operations Squadrons (SOS), Special Tactics Squadrons (STS), Special Operations Theater Air Operations Squadrons (SOTAOS), Special Tactics Training Squadrons (STTS), and those assigned to a SOFME UTC, will comply with AFI 44-103, *The Air Force Independent Duty Medical Technician Program*, IDMT Medical and Dental Treatment Protocols, IDMT User guide, and Comprehensive Medical Readiness Program (CMRP) requirements. They are also required to obtain the 455 SEI IAW 4N0X1 CFETP, and National Registry Paramedic (NRP) 4N0X1H.

4.9.2. SOIDMTs will be required to attend SOCMID Advanced Medical and Trauma Skills Sustainment Course (AMTSSC) to maintain NRP currency, and trauma skills sustainment requirements outlined in *USSOCOM DIR 350-29, Special Operations Forces Baseline Interoperable Standards for Medical Training, Dated 05 JAN, 2021*.

4.10. AFSOC Independent Duty Medical Technicians without a Co-located AF Medical Treatment Facility.

4.10.1. There are a number of AF IDMTs located within units without a co-located AF MTF. This has resulted in challenges with oversight of the IDMT program at these locations and difficulties for the IDMTs to complete and document training requirements with appropriate preceptor oversight.

4.10.2. The following guidance applies to AFSOC IDMTs within units without a co-located AF MTF and fulfills the requirement, per AFI 44-103, *The Air Force Independent Duty Medical Technician Program*, for the MTF Operating Instructions for these IDMTs:

4.10.2.1. Preceptors and functional area representatives approved for IDMT training will be appointed by the AFSOC IDMT Certification Authority or by the AFSOC/SGP.

4.10.2.2. The host MTF SGP will function as the SGH and the senior embedded 4N0X1X will function as the IDMT Coordinator. AFSOC/SG will have overarching oversight by monitoring the Executive Activity and Clinical Currency reports and will assume those duties in the event of vacancies for those positions.

4.10.2.3. Training requirements for IDMTs will be completed locally to the maximum extent practical. If training cannot reasonably be completed locally, coordinate with Medical Readiness Training (SGXT) to arrange for training options.

4.10.2.4. IDMTs covered in this policy will use the host MTF or Special Operations Medical Group (SOMDG) Pharmacy and Therapeutics Committee approved IDMT Authorized Drug List. Any changes or additions to this list will be approved by the AFSOC Chief of Aerospace Medicine (AFSOC/SGP).

4.10.2.5. IDMTs will report all BBP exposures IAW local IC and BBP guidance.

4.11. Special Operations Physical Therapist. Physical Therapists embedded in AFSOC operational units are neuromuscular experts who deliver care to optimize the performance of Air Commandos through direct access evaluation and treatment of neuromusculoskeletal disorders. They provide the full scope of physical therapy services and practice independently. Physical Therapists may perform joint manipulation, dry needling, battlefield acupuncture, and use other diagnostic and treatment modalities in accordance with their privilege list. Physical Therapists perform injury surveillance and collect data to inform injury prevention activities in group or individual settings. Physical Therapists consult with other medical providers and are responsible for communicating medical readiness status to line of the Air Force and A3M leadership. They collaborate with other human performance professionals including Athletic Trainers and Strength and Conditioning Coaches to ensure safe exercise progression and to prevent re-injury in members who have undergone a rehabilitative program. With the exception of prevention activities, Physical Therapists must document all clinical encounters and interventions in the member's electronic health record.

4.12. AFSOC Pararescueman (PJ)Medical Program. PJs are tasked to deliver effective emergency medical and trauma care in the operational environment at a paramedic level. NRP certification and Special Operations-Advanced Tactical Practitioner (SO-ATP) requirements IAW USSOCOM Directive 350-29. AFSOC PJs provide emergency medical treatment and lifesaving procedures at the paramedic level. NRP currency is required for service in the PJ 1Z1 AFSC, IAW AFMAN 10-3500V1, *Air Force Special Warfare Training Program*. PJs are combatants and thus are not protected as medical personnel by the Geneva Conventions or International Law.

4.12.1. PJs are not authorized to independently provide sick call medical care in garrison. However, PJs may work directly with the medical preceptor to diagnose and treat illnesses or injuries in a training capacity to develop and maintain clinical proficiency. While deployed, PJs may provide limited-scope Small Unit Care per the SO-ATP Tactical Medical Emergency Protocols (TMEPs) and if they meet the requirements for utilizing TMEPs IAW USSOCOM Directive 350-29.

4.12.2. PJ Medical Program Management. Overarching USAF PJ medical program management is contained in AFMAN 10-3509, *Air Force Special Warfare Medical Operations*. PJ medical training and qualification will be documented and maintained in the member's DAF Form 623, *Individual Training Record Folder* and IAW AFMAN 10-3500V1, Chapter 7. For convenience, medical training records may be maintained geographically separated from the DAF Form 623 record; refer to AFMAN 10-3500V1 for guidance.

4.12.3. HAF/A3S is the lead agency for Special Warfare and will appoint an Air Force PJ Medical Director IAW AFMAN 10-3509, *Air Force Special Warfare Medical Operations*. The Air Force PJ Medical Director addresses medical qualifications, requirements, and PJ medication, and PJ Medical Operations Handbook issues. Changes to training, protocols, medications, and equipment are coordinated through the Air Force PJ Medical Operations Advisory Board (MOAB) which meet twice yearly. AFSOC/SGP, ST medical personnel, and PJ representatives may attend annual MOABs.

4.12.4. Medical Director and Medical Control. The ST unit medical director is responsible for medical oversight of assigned PJs. The medical director will be a residency trained, board certified or board eligible credentialed physician appointed in writing by the ST unit commander. The requirement for residency trained physician may be waived by the AFSOC/SGP. A copy of the appointment letter will be forwarded to host base MTF SGH and to the AFSOC/SGP. Additionally, a copy will be filed in each PJ's medical training record. If the ST medical director is not present during deployed operations, the senior AFSOC flight surgeon in the AOR is responsible for AFSOC PJ medical oversight. If the senior AFSOC flight surgeon in the AOR is unfamiliar with PJ protocols or scope of medical practice, they will consult the STG/SG.

4.12.4.1. DOD, Department of Transportation, and USAF directives mandate appointment of a physician medical director to oversee the medical training, qualification, and practice of NRPs to include PJs. The unit PJ medical director and deployed flight surgeons providing deployed medical oversight must have expertise in operational medicine, trauma medicine, delivery of medical care in field environments, wilderness medicine, pre-hospital medical care, and medical transport/evacuation systems and equipment.

4.12.4.2. The medical director will provide clinical supervision, oversight, and guidance to the ST unit Non-Commissioned Officer of medical training (NCOMT).

4.12.4.3. In conjunction with unit standardization and evaluation team, the medical director will review and audit PJ medical training status to ensure PJ medical currency. Additionally, the medical director will conduct unannounced medical proficiency exercises and evaluations to ensure PJs and other appropriate ST personnel meet medical and applicable TCCC proficiency standards. The medical director will assist in planning and conducting PJ Medical Situation Exercises (MEDEX) and Medical Evaluations.

4.12.5. PJs assigned above squadron level (e.g., Group, Wing, or Higher Headquarters) will fall under the medical direction of the local ST squadron medical director.

4.12.6. NCO of Medical Training (NCOMT). The NCOMT will be designated by the unit commander in writing and will manage the PJ medical program. The NCOMT will use AFMAN 10-3500V1, Attachment 2, Training Requirements and AFMAN 10-3509, *Air Force Special Warfare Medical Operations* as medical program guidance. The NCOMT must work closely with the medical director and senior team PJ to produce a robust PJ medical program.

4.12.6.1. The NCOMT should be a fully qualified IDMT, preferably with an operational background. The NCOMT should have at least two years of experience as an IDMT and should be an experienced Basic Life Support (BLS) and TCCC instructor.

4.12.6.2. The NCOMT's secondary duties will include conducting required medical classes and training for unit operators and personnel. Additionally, the medical director and NCOMT will provide emergency medical coverage for unit training.

4.12.7. Operational Medical Guidance. AFSOC PJs will follow approved published guidance for medical treatment protocols to include the Pararescue Medical Operations Handbook, USSOCOM TMEPs, and additional appropriate paramedic level pre-hospital continuum of care as defined in current paramedic textbooks.

4.12.8. Operational Medical Reporting. PJs will complete Guardian Angel (GA) Consolidated Mission Reports (CMRs) for all missions. Medical treatment information will be extracted for submission and database input.

4.12.8.1. The ST medical director or deployed AFSOC flight surgeon will submit the GA CMR to the Air Force PJ Medical Director, SOW/A3Ms, and AFSOC/SGP.

4.12.8.2. Ensure submission of the GA CMR to AFSOC/A3S, Command Personnel Recovery PJ Superintendent occurs via appropriate secure communication channels.

4.12.9. PJ Medical Qualification. The following qualifications and certifications are mandatory requirements for operational PJs assigned to AFSOC:

4.12.9.1. Current NRP certification and all associated certification requirements, including BLS, Pediatric Advanced Life Support, and ACLS.

4.12.9.2. AFSOC PJs will follow SOCOM training requirements as specified in USSOCOM Directive 350-29.

4.12.10. Non-standard medications. Non-standard medications are defined as medications administered by PJs that are not included within the Pararescue Medical Operations Handbook or the USSOCOM TMEPs. Any administration of non-standard medications by PJs must be strictly IAW DOD, USAF and USSOCOM policy, and must be justified by mission requirements. Additionally, non-standard medication administration must be approved by the AF PJ medical director prior to use. Appropriate training must be completed and documented prior to PJ administration of non- standard medications. If non-standard medication training is completed for PJs while deployed, the training must be documented and a copy of the documentation must be forwarded to the Air Force PJ Medical Director and AFSOC/SGP, the PJ's home station medical director, and NCOMT for placement in the PJ's medical training record. If non-standard medication use is formally approved, the PJ who administers it will report this (include patient's name, age, medication, dosage and indications) to the supervising/authorizing physician, ST medical director, SOW/A3M, ACC/SGP, and AFSOC/SGP.

4.12.11. Experimental medical materials and equipment. Use of experimental medical materials and equipment is not authorized unless formally approved through standard DOD, USAF, USSOCOM, and AFSOC institutional research board processes.

4.13. Special Operations Medical Logistics.

4.13.1. The AFSOC Medical Logistics Technician/4A1XX Functional Manager is responsible for the organization, training, and the coordination of 4A1 personnel assignments to AFSOC operational units. The 4A1XX functional will ensure compliance with AF and AFSOC instructions/policies.

4.13.1.1. All AFSOC Medical Logistics Technicians will attend at least 12 hours of documented Readiness Skills Verification Training annually. If training cannot be accomplished at the local MTF, in-service training is acceptable with prior approval from the 4A1XX Functional Manager.

4.13.1.2. AFSOC Medical Logistics Technicians require additional training above and beyond conventional force Logistics Technicians. See AFSOCI48-1010 *Supplement, Medical Training Tables,* Chapter 4 for complete Medical Logistician Training Plan. A 3 year controlled tour is strongly recommended.

4.13.2. AFSOC/SGXL is responsible for Manning and Equipment Force Packaging (MEFPAK) related requirements. Any proposed changes to MEFPAK allowance standards will be routed to AFSOC/SGXL.

4.13.3. Medical Logistics technicians are responsible for unit WRM and associated medical logistics functions IAW AFMAN 41-209, paragraph 8.25 – Use of Medical WRM. Changes to medical logistics related policies/procedures will be coordinated with AFSOC/SGXL and approved by AFSOC/SG.

4.13.4. Embedded logistics may use all WRM assets for JCET missions, Humanitarian Assistance, Disaster Relief missions, Joint Chiefs of Staff directed exercises. Tasking memorandum or memorandum of understanding with appropriate line of accounting will be routed to AFSOC/SGXL.

4.13.5. The embedded AFSOC Medical Logistics Technician with assistance from the in-garrison contractors is responsible for inventorying WRM no less than 24 months from the previous inventory. IAW AFMAN 41-209, the due date for inventory completion is the final calendar day of the anniversary month.

4.13.5.1. An inventory is not considered closed until all actions outlined in AFMAN 41-209 paragraph 8.16.3.7. are completed and documented.

4.13.5.2. A signed copy of the Inventory Summary Report will be forwarded to the 4A1XX Functional Manager.

4.14. Operational Bioenvironmental Engineering (BEE) Personnel Roles and Responsibilities.

4.14.1. Operational BEE personnel will train OCS personnel on BEE tactics, techniques, and procedures. Training will include familiarity with the Water Protection and Industrial Hygiene Programs. BEE personnel will also serve as reach-back support for deployed members.

4.14.2. Operational BEE personnel will maintain their skills and proficiency by annually coordinating tasks listed below with the host MTF.

4.14.2.1. One routine industrial hygiene survey including Defense Occupational and Environmental Health Readiness System (DOEHRS) entry.

4.14.2.2. One water sampling survey.

4.14.2.3. One special survey.

4.14.2.4. Participate in an emergency management exercise.

4.14.2.5. Maintain 100% currency on Respiratory Syncytial Virus (RSV) program.

4.14.2.6. Participate in the equipment proficiency analytical testing program. If equipment is not available to complete proficiency test, submit a waiver request to AFSOC/SGPB for a suitable alternate test.

4.14.2.7. Attend host MTF's Occupational and Environmental Health Working Group meeting when AFSOC specific issues or work-centers are discussed**.**

4.14.3. Quarterly training updates for annual requirements will be documented and emailed to AFSOC/SGPB and the 4B AFSOC Functional Manager no later than 15 days after the end of each quarter.

4.14.4. Operational BEE personnel will be the OPR for performing BEE support to the OCS for all elements relating to the Industrial Hygiene Program. Responsibilities and tasks will be transferred to the host Medical Group or installation BEE equivalent offices when there is a conflict with AFSOC mission operations after coordination with AFSOC/SGPB and host MTF BEE Flight.

4.14.5. Operational BEE personnel will conduct Occupational and Environmental Site Assessments at deployed locations as needed IAW DAFMAN 48-146, *Occupational Health Program Management.* Information collected will be used to generate the Occupational and Environmental Health Exposure Data and/or a Periodic Occupational and Environmental Measurement Summary. Operational BE personnel will coordinate with Public Health to ensure exposure data is incorporated into personnel medical records as required.

4.15. Operational Public Health Personnel Roles and Responsibilities.

4.15.1. Act as the primary Public Health trainer for OCS personnel. Public Health personnel will also serve as reach-back support for deployed members.

4.15.2. Maintain current medical threat and countermeasure briefs for the OCS mission IAW Defense Health Agency Procedural Instruction (DHA-PI) 6490.03, *Deployment Health Procedures* and the Combatant Commander's (COCOMs) reporting instructions.

4.15.3. Act as the primary liaison for unit personnel regarding the Medical Employee Health Program, BBP program, and communicable diseases. Unit is defined as any AFSOC or USSOCOM unit outside of a MTF with assigned Public Health personnel.

4.15.4. Request host MTF 4E Functional review of their training record on a quarterly basis. If the local 4E Functional is not available, OCS Public Health personnel will send the request to AFSOC 4E MAJCOM Functional Manager.

4.15.5. Maintain AFSC and CMRP proficiency by coordinating the training or tasks listed below with the host Medical Group. To the maximum extent possible, tasks, skills, or surveys should be aligned with the SOW or Special Operations Group (SOG) Public Health requirements.

4.15.6. Attend host MTF Occupational and Environmental Health Working Group meetings when AFSOC specific occupational health issues or work-centers are discussed.

4.15.7. Maintain 100% currency for all CMRP training/tasks. Work with the host Medical Group to accomplish any training gaps.

4.15.7.1. Complete one food facility inspection every 6 months. This requirement may also be accomplished while deployed or attending an exercise.

4.15.7.2. Complete one public facility inspection every 12 months.

4.15.7.3. Complete one entomology surveillance every 12 months. This requirement may also be accomplished while deployed or attending an exercise.

4.15.7.4. Complete one ration inspection every 12 months.

4.15.7.5. Complete one Occupational Health Risk Assessment every 12 months.

4.15.7.6. Complete one water sampling survey every 12 months.

4.15.7.7. Participate in emergency management exercises with biological scenarios.

4.15.7.8. Complete all AFSOC training, as prescribed in the Training Tables.

4.15.7.9. Operational Public Health personnel will be the OPR for performing Public Health support to the OCS for all elements relating to Force Health Protection activities. Responsibilities and tasks will be transferred to the host Medical Group or installation Public Health equivalent offices when there is a conflict with AFSOC mission operations after coordination with AFSOC/SGPM and host MTF Public Health Flight.

4.15.7.10. Operational Public Health personnel will support Occupational and Environmental Site Assessments at deployed locations as needed. Information collected will be used to generate the Occupational and Environmental Health Exposure Data and/or a Periodic Occupational and Environmental Measurement Summary. Operational Public Health personnel will coordinate with BEE to ensure exposure data is incorporated into personnel medical records as required.

4.16. Vendors and Products.

4.16.1. Operational medical personnel should refer all vendor requests for product test & evaluation, feedback and/or sponsorship (e.g., Small Business Innovation Research [SBIR] customer memorandums) to AFSOC/SGR. This will allow one point of contact (SGR) to ensure the most appropriate vendor is being used and all legal and safety requirements (e.g., Safe-to-Fly) are met. SGR involvement in this process will also reduce redundant effort and enable successful products to transition to alternate funding streams for commercialization and/or inclusion in allowance standards.

4.16.2. Use of any medical products and procedures not approved by the Food and Drug Administration requires SGR to conduct user training when appropriate, in accordance with applicable FDA Emergency Use Authorization requirements.

Chapter 5

TRAINING

5.1. Training Requirements. AFSOC operational medical personnel require training to a level far exceeding their non-SOF colleagues in order to successfully operate in the SOF environment and support USSOCOM missions.

5.1.1. AFSOC Medical UTC Training and Clinical Currency Requirements are outlined in the Medical Training Tables located at: **https://usaf.dps.mil/sites/AFSOC-SG/SGX/SitePages/Home.aspx**

5.1.2. AFSOC/SGX is the OPR for any changes to the Medical Training Tables.

5.1.3. Medical Readiness Reporting. All AFSOC medical personnel must document medical readiness requirement completion IAW AFI 41-106, *Medical Readiness Program,* AFI 10-201, *Force Readiness Reporting*, and AFSOCI48-1010 *Supplement, Medical Training Tables,* in order to maintain accurate medical readiness reporting in MRDSS and DRRS. Reports will be sent to the organizational SIPR email at **afsoc.sgx@socom.smil.mil**.

5.2. Administration of Medical Training. AFSOC/SG is responsible for validating medical training requirements and clinical currency requirements. Unit commanders will ensure assigned operational medical personnel complete all training requirements.

5.2.1. Medical Training Manager (MTM). All AFSOC unit commanders with medical personnel assigned will appoint an MTM to serve as the POC for medical training.

5.2.2. MTM appointment criteria, roles, and responsibilities are outlined in **Chapter 1** of the Medical Training Tables.

5.3. Formal Training. AFSOC/SGXT manages the medical training seats and coordinates with unit commanders and/or appointed MTMs to fill training seats with members (to include ANG and AFRC) who require UTC and AFSC specific training. AFSOC/SGXT will monitor MRDSS on a regular basis and will report, in writing, any discrepancies or noncompliance to AFSOC/SG and the appropriate unit commander.

5.3.1. Formal Training Cancellation. In the event of a canceled formal training that results in the loss of a SORTS-reportable training seat, MTM will provide a cancellation letter signed by the member's flight commander and include course name, dates and location with member's name, rank, AFSC, and justification for cancelation.

5.3.2. Justification letter will be sent to AFSOC/SGXT. Template found in **Attachment 5**.

5.3.3. Formal Training Waivers. Requirements apply to all operational medical personnel. If newly assigned personnel have previously served in a similar position in an AFSOC unit, portions of stated mission qualification requirements may be waived by AFSOC/SG on an individual basis.

5.3.4. Waiver requests will contain the following: member's name, rank, position, unit, reason member already meets requirement(s), justification for waiver, and impact if waiver disapproved, (**Attachment 5**). Waiver requests must be endorsed by member's squadron and group commander.

5.4. AFSOC Medical Training. All newly assigned AFSOC operational medical personnel are required to successfully complete all trainings outlined in Medical Training Tables. Regardless of previous assignment, ALL operational medical personnel are required to complete Special Operations Forces Medical Skills Development (SOF MSD) at the Tactical Operational Medical Skills (TOMS) Lab for their baseline training.

5.4.1. Special Operations Command Medical Officer Special Experience Identifier (SEI).

5.4.2. While not mandatory, all AFSOC Medical Officers are encouraged to pursue the SEI for their career field. Refer to the Air Force Officer Classification Directory (AFOCD) for requirements.

5.5. Approved Training Platforms.

5.5.1. Trauma Training. Trauma Skills Sustainment and trauma related Continuing Medical Education (CME) requirements are outlined for each UTC in the Medical Training Tables.

5.5.2. Trauma Skills Sustainment. In addition to clinical competency and clinical currency requirements, operational personnel require periodic exposure to hands-on human trauma patient management in a controlled environment where current best practices in trauma care can be refreshed. AFSOC personnel who are unable to meet the American College of Surgeons' recommendation to operate and manage 35 multi-trauma patients per year must attend Center for Sustainment of Trauma and Readiness Skills (C-STARS) or a Sustained Medical and Readiness Training course at a Regional Currency Site (SMART-RCS) every 24 months. AFSOC personnel who meet the 35 multi-trauma resuscitation requirements a year, in addition to further requirements for certain AFSCs, will follow the 24-month C-STARS exemption process as outlined in AFI 41-106, *Air Force Medical Readiness Program*. Approved methods for acquiring additional trauma patient experience include:

5.5.2.1. While deployed, appropriately documented trauma patient management may be counted for up to 50% of this trauma skills sustainment requirement. The documented trauma patient care must be subject to, and available for, the formal peer-review process and be approved by AFSOC/SGP, then filed in the individual's training folder.

5.5.2.2. Graduate Medical Education. Members reporting to AFSOC directly from residency training programs meet this trauma skills sustainment requirement if member successfully completed the program and are assigned to AFSOC within two years from the last routine exposure to significant numbers of trauma patients in the setting of trauma surgery, emergency department at a trauma center, or anesthesia at a trauma center. Members reporting to AFSOC directly from an internship-only training program must be evaluated by the SOW/SG and be approved by AFSOC/SGP, to determine if they will need immediate trauma skills sustainment training at C-STARS.

5.5.3. **Special Operations Center for Integration and Development.** SOCMID is an approved platform for 4N0X1C and Pararescuemen Trauma Skills Sustainment Training.

5.6. Mission Qualification Training. This section establishes the minimum medical training requirements to attain initial mission qualified status for AFSOC operational medics, beyond the AFMAN 48-149 Military Qualifications Upgrade Training (MQT) requirements.

5.6.1. Operational medical personnel are mission qualified after completing formal AFSC awarding courses/programs, CMRP requirements, and all training outlined in Medical Training Tables. Members must be compliant with Clinical currency requirements as shown in the Medical Training Tables prior to deployment. AFSOC/SG may delegate waiver authority to AFSOC/SGP.

5.6.2. SOTAOS medical personnel are mission qualified after completing applicable AFSC courses, training requirements outlined in Medical Training Tables, and all additional training required by SOTAOS/CC. Members must be compliant with clinical currency requirements as shown in the Medical Training Tables prior to deployment or conducting theater engagements. AFSOC/SG may delegate waiver authority for IHS or Air Advisor related requirements to AFSOC/SGXK.

5.6.3. SOST personnel are mission qualified after completing formal assessment and selection, AFSC specific medical or surgical training programs, CMRP training, and training requirements outlined in Medical Training Tables IAW 720STG OI 10-3529 Vol1, *Special Operations Surgical Team Training*. SOST personnel must be in good standing with their assigned medical institution and must be clinically competent and current to retain mission qualification.

5.6.3.1. SOST personnel will maintain clinical competency and clinical currency IAW requirements/guidelines set by AFMS, functional area managers, AFMS corps chiefs, AF/SG specialty consultants, applicable professional standards to include licensure, board certification, continuing medical education requirements, and those contained herein. General Surgeons, Anesthesiologists, and Emergency Medicine Specialists are expected to obtain and maintain specialty Board Certification. Certified Registered Nurse Anesthetists, Critical Care Nurses, Emergency Trauma Nurses, Surgical Technicians and Respiratory Care Practitioners will maintain all certifications and a robust clinical schedule.

5.6.3.2. SOST clinical currency requirements are listed by AFSC in the Medical Training Tables. Clinical Currency Tables outline the minimum requirement; additional clinical experience is encouraged to maintain high proficiency. Credit for clinical currency caseloads will be pro-rated when a surgeon is deployed to include approved reconstitution time. AFSOC/SG may delegate waiver authority to AFSOC/SGO.

5.7. Special Operations Surgical Team (SOST) Unit Type Code (UTC) Training. SOST Initial Training will be conducted IAW this instruction, 720STGOI 10-3529 Vol1, and Medical Training Tables

5.8. Aeromedical Physician Assistant and Aeromedical Nurse Practitioner Training.

5.8.1. AFSOC APAs and ANPs will comply with training and recertification requirements as defined by AFI 44-102 and AFMAN 48-149.

5.8.2. Operational APAs and ANPs assigned to line units must accomplish Principles of Aerospace Medicine (PoAM), Aerospace Medicine Primary (AMP) 1, AMP 2, AMP 3, & Aircraft Mishap Investigation and Prevention (AMIP) courses as soon as practical upon notification of assignment, but no later than 1 year after arrival at the unit. APAs and ANPs must complete applicable courses prior to practicing aerospace medicine.

5.8.3. AFSOC APAs and ANPs assigned as operational medical personnel will receive clinical oversight by the SOW/SG in their respective units for six months per AFMAN 48-149 and coordinate with the host MTF SGH to update their privilege list and document the appropriate scope of practice IAW AFI 44-119.

5.9. Special Operations Independent Duty Medical Technician (SOIDMT) Training.

5.9.1. SOIDMTs will comply with all training and recertification requirements outlined in AFI 44-103, 4N0X1C CFETP SEI 455, *Special Operations Command Medic*, USSOCOM Directive 350-29, and all requirements outlined within supplements of this instruction.

5.9.2. SOIDMT Medical Training Plan. Requirements for obtaining the 455 SEI and to be considered fully mission qualified are outlined in the Medical Training Tables, **Chapter 4**

5.10. Pararescuemen Medical Training. AFSOC PJs will maintain National Registry of Paramedic certification and SO-ATP training as directed by USSOCOM 350-29. PJ continuing medical education and training report requirements described in AFMAN 10-3500V1 will be accomplished IAW applicable training tables.

5.10.1. The following certifications, qualifications, evaluations and training are mandatory for AFSOC PJs:

5.10.1.1. NRP certification is required during initial PJ training. This certification is obtained prior to the first phase of the 3-level PJ apprentice course.

5.10.1.2. NRP core requirement re-certification is required every two years. Completion of the SOCMID or AFSOC/SG approved alternative such as Joint Special Operations Medical Training Center (JSOMTC) Special Operations Combat Medic Skills Sustainment Course is mandatory for AFSOC PJs.

5.10.1.3. Patient encounters and Trauma Skills Sustainment Training are recommended every two years. SOCMID manages Advanced Medical and Trauma Skills Sustainment Course which includes clinical rotations at University of Alabama (UAB) Hospital and continuing education in accordance with USSOCOM Special Operations Forces Basic Interoperability Standards

5.10.1.4. Missions completed in the execution of combat, military, civil, or humanitarian operations may be used towards patient encounter and trauma sustainment currency. Documented hospital emergency and trauma department performance may also count towards meeting currency requirements.

5.10.1.5. Refer to DoD Instruction 1322.24, *Medical Readiness Training* (MRT), AFMAN 10-3500V1 and USSOCOM Dir 350-29 for training frequency.

5.10.1.6. Medical Skills Certifications Requirements. PJs will perform MEDEX and Medical Skills Certifications Requirements in compliance with the latest AFMAN 10-3500V1, *Air Force Special Warfare Training Program* requirements.

5.10.2. Unit lectures and CME. Unit medical directors are encouraged to conduct routine medical classes, lectures, and seminars for unit PJs. Furthermore, unit medical directors will schedule/coordinate PJ medical training by guest physicians and specialists. Unit CME hours will be documented in PJ training records.

5.10.3. Additional medical skills instruction, validation, and continuum of practice certification will be documented and placed in the medical training records. This instruction includes topics such as blood administration/transfusion.

5.10.4. Clinic Sick Call. PJs may not provide independent medical care in out-patient clinic or sick call settings unless the situation is an emergency (life, limb, eyesight). However, PJs are authorized to apply PJ Medical Operations Handbook for Small Unit Care protocols SO-ATP under the direct supervision of a physician in the clinic. Care rendered by a PJ must be documented in the health record and signed by the precepting physician. STG/SG and unit Medical Directors will ensure clinical training is documented in PJ medical training records and directly supervised by a physician. Medical skills training documentation must include operating parameters, re-qualification or currency standards, and a skill practice expiration date.

5.11. AFSOC Multi-Modal Patient Movement (MMPM) Training.

5.11.1. Refer to the Medical Training Tables for MMPM training requirements and frequency.

5.11.2. The only approved platform for MMPM initial training is the TOMS Lab. Waiver authority for alternative MMPM courses is AFSOC/SGP.

5.11.3. MMPM sustainment and currency will be maintained by completing one training or real-world inflight MMPM mission with an evaluation, every six months. More frequent hands-on, scenario-based training is highly encouraged. In the instance of no aircraft availability, training may be accomplished on a static airframe and/or a training simulator. All MMPM training, simulated or inflight, must be accomplished IAW appropriate UTC Mission Capability (MISCAPs) and Tactics Techniques and Procedures (TTPs). Enlisted members will document this training in MyLearning and MRDSS. Officers will document this training in their Competency Assessment Folder and MRDSS. Noncurrent members will be evaluated for proficiency regarding MMPM equipment and procedures using appropriate standards of medical care, and the approved MMPM checklist.

5.11.4. Operational medical personnel will maintain weapons qualifications, field craft, and tactical skills commensurate with assigned UTC. SOFME, SOST, SOTAOS, and Special Tactics medical personnel may participate in live fire training exercises in order to integrate medical training requirements with aviation and special warfare operations. Weapons qualifications/training will be IAW DAFI 31-131, *Combat Arms Training and Maintenance (CATM) Program*, and will be consistent with the member's Geneva Conventions status. Unit commanders are responsible for ensuring personnel sustain these skills.

5.12. Advanced Medical Training (AMT). For guidance on AMT request and approval process, request documents at the Controlled Unclassified Information level via NIPR to AFSOC/SGO at **AFSOC.SGO@us.af.mil**.

5.12.1. All AMT will meet the requirements prescribed in USSOCOM Directive 350-29, Medical Special Operations Forces (SOF) Baseline Interoperable Standards. For AFSOC AMT events, Initial Notification Request must be sent to **AFSOC.SGO@us.af.mil** NLT 30 days prior to training for CONUS events and NLT 90 days prior for OCONUS events. For TSOC led AMT events, the TSOC/SG will notify USSOCOM Veterinary Review Office (VRO) **socom_vet@socom.mil**,of AMT events NLT 60 days prior to event and will follow-up monthly until event is conducted or canceled.

5.12.2. Units will make the maximum use of didactic, moulage, mannequin, simulator, and other training alternatives.

MELISSA J. DOOLEY, Colonel, USAF, MC, FS
Command Surgeon, Air Force Special Operations Command

Attachment 1

GLOSSARY OF REFERENCES AND SUPPORTING INFORMATION

References

720STG OI 10-3529 Vol 11, *Special Operations Surgical Team Training,* 28 February 2023

AFI 10-1302, *Air Force Lessons Learned Program*, 30 July 2019

AFI 10-201, *Force Readiness Reporting,* 5 June 2024

AFI 10-3504, *Dive Program*, 28 October 2024

AFI 10-402, *Mobilization Planning, 8 March 2018*

AFI 33-322, *Records Management and Information Governance Program*, 23 March 2020

AFI 41-106, *Medical Readiness Program*, 29 July 2020

AFI 44-102, *Medical Care Management*, 17March 2015

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USSOCOM Directive 350-29, *Special Operations Forces Baseline Interoperable Medical Training Standards*, 5 January 2021

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Prescribed Forms

None

Adopted Forms

AF Form 1041, *Medical Recommendation for Flying or Special Operational Duty Log*

AF Form 579, *Controlled Substances Register*

DAF Form 623, *Individual Training Record Folder*

AF Form 847, *Recommendation for Change of Publication*

DD Form 2992, *Medical Recommendation for Flying or Special Operational Duty*

Abbreviations and Acronyms

AAR—After Action Report

ACC—Acute Care Clinic

ACLS—Advanced Cardiac Life Support

ACS—Aeromedical Consultation Service

AEF—Air and Space Expeditionary Force

AF—Air Force

AFIA—Air Force Inspection Agency

AFI—Air Force Instruction

AFMAN—Air Force Manual

AFMEDCOM—Air Force Medical Command

AFMS—Air Force Medical Service

AFOCD—Air Force Officer Classification Directory

AFOMIS—Air Force Operational Medicine Information System

AFRC—Air Force Reserve Command

AFRIMS—Air Force Records Information Management System

AFSC—Air Force Specialty Code

AFSOC—Air Force Special Operations Command

AFTTP—Air Force Tactics, Techniques, and Procedures

AHLTA-T—Armed Forces Health Longitudinal Technology Application-Theater

AIMWTS—Aeromedical Information Management Waiver Tracking System

ALS—Advanced Life Support

AMIP—Aircraft Mishap Investigation and Prevention

AM&E—Assessment Monitoring & Evaluation

AMRO—Airman Medical Readiness Optimization

AMP—Aerospace Medicine Primary

AMT—Advanced Medical Training

AMTSSC—Advanced Medical and Trauma Skills Sustainment Course

ANG—Air National Guard

ANP—Aeromedical Nurse Practitioner

AOME—Aerospace and Operational Medicine Enterprise

AOR—Area of Responsibility

APA—Aeromedical Physician Assistant

ARC—Air Reserve Components

ART—Airman Resiliency Teams

ASIMS—Aerospace Information Management System

ATLS—Advanced Trauma Life Support

AvIP—Aviation Incentive Pay

BATDOK—Battlefield Assisted Trauma Distributed Observation Kit

BBP—Blood Borne Pathogen

BEE—Bioenvironmental Engineering

BLS—Basic Life Support

BOMC—Base Operation Medicine Clinic

C2—Command and Control

CBRN—Chemical, Biological, Radiological, Nuclear

CHAMP—Consortium for Health and Military Performance

CFETP—Career Field Education and Training Plan

CME—Continuing Medical Education

CMR—Consolidated Mission Reports

CMRP—Comprehensive Medical Readiness Program

COCOM—Combatant Commanders

C-STARS—Center for Sustainment of Trauma and Readiness Skills

DAFPD—Department of the Air Force Policy Directive

DAFMAN—Department of Air Force Manuals

DCR/DCS—Damage Control Resuscitation/Damage Control Surgery

DES—Disability Evaluation System

DHA—Defense Health Agency

DHA-PI—Defense Health Agency Procedural Instruction

DHA-PM—Defense Health Agency Procedural Manual

DHP—Defense Health Program

DMO—Diving Medical Officer

DNBI—Disease Non-Battle Injury

DNIC—Duties Not Including Controlling

DNIF—Duties Not Involving Flying

DOD—Department of Defense

DODI—Department of Defense Instruction

DOEHRS—Defense Occupational and Environmental Health Readiness System

DRHA—Deployment Readiness Health Assessment

DRRS—Defense Readiness Reporting System

EHR—Electronic Health Record

EUD—End User Device

FDA—Food and Drug Administration

FOMC—Flight and Operational Medical Clinic

FOMP—Flight and Operational Medicine Program

FS—Flight Surgeon

GA—Guardian Angel

GHE—Global Health Engagement

HADR—Humanitarian and Disaster Relief

HARM—Host Aviation Resource Management

HCD—Health Care Delivery

HCP—Healthcare Provider

HIPAA—Health Insurance Portability and Accountability Act

HIV—Human Immunodeficiency Virus

HPW—Human Performance Wing

IAW—In Accordance With

IC—Infection Control

IDMT—Independent Duty Medical Technician

IHS—International Health Specialist

IMA—Individual Mobilization Augmentee

IMR—Individual Medical Readiness

IPR—Installation Personnel Readiness

IRB—Institutional Review Board

IT—Information Technology

IRON—Integrated Resiliency Optimization Network

JCET—Joint Combined Exchange Training

JOMIS—Joint Operational Medicine Information Systems

JSOMTC—Joint Special Operations Medical Training Center

JTS—Joint Trauma System

Kx—Knowledge Exchange

MAJCOM—Major Command

MASCAL—Mass Casualty

MDD—Medication Dispensing Devices

MDG—Medical Group

MEB—Medical Evaluation Board

MEDCOP—Medical Common Operating Picture

MEDEX—Medical Situational Exercise

MEFPAK—Manning and Equipment Force Packaging

METL—Mission Essential Task List

MHS-G—Military Health Systems-Genesis

MHSG-T — MHS Genesis—Theater

MICT—Management Internal Control Toolset

MISCAP—Mission Capability

MMPM—Multimodal Patient Movement

MOAB—Medical Operations Advisory Board

MQT—Military Qualification Upgrade Training

MRDSS—Medical Readiness Decision Support System

MRT—Medical Readiness Training

MSC—Medical Service Corps

MTF—Military Treatment Facility

MTM—Medical Training Manager

NCOMT—Non-Commissioned Officer of Medical Training

NRP—National Registry EMT Paramedic

O&M—Operation and Maintenance

OCS—Operational Clinical Services

OFDA—Operational Flying Duty Accumulator

OMDS—Operational Medical Data Service

OpMed—Operational Medicine

OpMed CDP—Op Med Care Delivery Platform

OPR—Office of Primary Responsibility

Ops Psych—Operational Psychologist

OPSS—Operation Supplemental Safety

ORM—Operational Risk Management

OSM—Operation Support Medical

PCC—Prolonged Casualty Care

PE—Performance Enhancement

PEBLO—Physical Evaluation Board Liaison Officer

PEP—Post-Exposure Prophylaxis

PHA—Periodic Health Assessment

POI—Point of Injury

PJ—Pararescuemen

PoAM—Principles of Aerospace Medicine

POTFF—Preservation of the Family and Force

PPE—Personal Protective Equipment

QA—Quality Assurance

RIO—Readiness Integration Office

RDS—Records Disposition Schedule

RPA—Remotely Piloted Aircraft

RSV—Respiratory Syncytial Virus

SERE—Survival, Evasion, Resistance and Escape

SGH—Chief, Medical Staff

SGPA—Chief, Flight Medicine

SGP—Chief of Aerospace Medicine

SG—Surgeon General, Command Surgeon

SGX—Chief, Medical Readiness Division

SGXL—Deputy, Medical Readiness

SGXT—Medical Readiness Training

SIB—Safety Investigation Board

SITREP—Situation Report

SMART-RCS—Sustained Medical and Readiness Training Course Regional Currency Site

SO-ATP—Special Operations Advanced Tactical Practitioner

SOCMID—Special Operations Center for Medical Integration and Development

SOFME—Special Operations Forces Medical Element

SOFMSD—Special Operations Forces Medical Skills Development

SOF—Special Operations Forces

SOG—Special Operations Group

SOIDMT—Special Operations Independent Duty Medical Technician

SOMDG—Special Operations Medical Group

SORN—System of Records Notice

SORTS—Status of Resources and Training System

SOS—Special Operations Squadron

SOST—Special Operations Surgical Team

SOTAOS—Special Operations Theater Air Operational Support

SOTG—Special Operations Tactic Group

SOW—Special Operations Wing

SQ/CC—Squadron Commander

STG/SG—Special Tactics Group Surgeon

STG—Special Tactics Group

STO—Special Tactics Officer

STS—Special Tactics Squadron

STTS—Special Tactics and Training Squadron

SWA—Special Warfare Airman

SW—Special Warfare

TBLD-M—Theater Blood Mobile

TC2—Theater Composite Health Care System

TCCC—Tactical Combat Casualty Care

TMDS—Theater Medical Data Store

TMEP—Tactical Medical Emergency Protocols

TMIP—Theater Medical Information Program

TN—True North

TOMS—Tactical Operational Medical Skills

TOCO—Theater Special Operations Center

TTP—Tactics, Techniques & Procedures

UAB—University of Alabama at Birmingham

UMO—Underwater Medical Officer

USSF—United States Space Force

USSOCOM—United States Special Operations Command

UTC—Unit Type Code

VRO—Veterinary Review Office

WRM—War Reserve Materiel

Office Symbols

AFSOC/A3—Director of Operations

AFSOC/A3S—Directorate Special Warfare

AFSOC/A1Z—Integrated Resilience Division

AFSOC/SG—Command Surgeon

AFSOC/SGO—Medical Operations Division

AFSOC/SGP—Aerospace Medicine Division

AFSOC/SGR—Research Division

AFSOC/SGPB—Bioenvironmental Branch

AFSOC/SGPM—Public Health Branch

AFSOC/SGX—Medical Readiness Division

AFSOC/SGXK—Global Health Engagement

HAF/A3S—HAF/Directorate Special Warfare

MDG/CC—Medical Group Commander

MDG/SGP—Medical Group Chief of Aerospace Medicine

MTF/CC—Medical Treatment Facility Commander

SOW/A3M—Medical Operations Division

SOW/SG—Surgeon General

SOW/CC—Commander

STG/CC—Special Tactics Group Commander

Attachment 2

AFSOC GO PILL AUTHORIZATION FOR USE

Figure A2.1. AFSOC Go Pill Authorization For Use.

AFSOC GO PILL AUTHORIZATION FOR USE

(SAMPLE)

Date: 1 Jan 20XX
From: Unit/CC

To: AFSOC/SGP

Subject: Fatigue Management Medication Use

The following personnel have been authorized the use of Dextroamphetamine/Modafinil for deployment operations:

	Last name,	First name	Unit	Projected Dates of Use (6 months maximum)
1.	Doe	John	XX	1 Jan XX to 1 Mar XX
2.				
3.				
4.				
5.				
6.				
7.				
8.				
9.				

I have verified that each individual listed above has been ground tested, cleared for use, and all documentation is in their medical record.

Signature Block

(Validating Flight Surgeon)

Use of fatigue management medications is approved for the personnel listed above. I certify the use of these medications is appropriate, and all other fatigue management tools have been considered and will be used to the maximum extent possible. Use is approved for ground operations, not for parachute or diving operations. The use of Dextroamphetamine/Modafinil is completely voluntary.

Signature Block

(Unit/CC)

Provide copy of this approval form to AFSOC/SGP, and home station Squadron.

Attachment 3

DEPLOYED QA OVERSIGHT DOCUMENTATION

Figure A3.1. Deployed QA Oversight Documentation.

DEPLOYED QA OVERSIGHT DOCUMENTATION

(SAMPLE)

Authority: 10 U.S.C. 55, *Medical and Dental Care*; 10 U.S.C. 8013, *Secretary of the Air Force*; and E.O 9397 Purpose: To document quality assurance reviews of chart reviews. Routine Uses: Internal review, no disclosure outside DOD. Disclosure: Voluntary, failure to provide requested information may result in delay of training requirements.										
1		2	3	4	5	6	7	8	9	10
Reviewer (Name, Unit)										
Lt Col Doe, XX SOW										
Provider (Name, Unit)	Date	Date	Date	Date	Date	Date	Date	Date	Date	Date
Capt Smith XX SOW	10/22									
	Pt Initials + last 4	Pt Initials + last 4	Pt Initials + last 4	Pt Initials + last 4	Pt Initials + last 4	Pt Initials + last 4	Pt Initials + last 4	Pt Initials + last 4	Pt Initials + last 4	Pt Initials + last 4
	jdr									
Chart Documentation	Y									
Prevention Addressed	Y									
Was Pain Assessed	Y									
If yes, was pain managed	Y									
Appropriate Antibiotic Usage	Y									
List Antibiotic	N									
Appropriate use of Radiology	Y									
Consult Utilization	Y									
Appropriate Medication Rx	N									
Lab Utilization	N/A									
Grounding Management	N/A									
Y = Yes - item used or ordered and met criteria; N = No - item did not meet criteria or was indicated and not used; N/A = Not applicable – not used or ordered and was not indicated										
Documentation of Discrepancies – All items marked "N" require Explanation										
Chart #	Discrepancy – Give brief explanation of how criteria are not met. Note if Discussed with provider									

Attachment 4

OPERATIONAL MEDICAL PERSONNEL EXECUTIVE ACTIVITY QUARTERLY REPORT

Figure A4.1. Operational Medical Personnel Executive Activity Quarterly Report.

OPERATIONAL MEDICAL PERSONNEL EXECUTIVE ACTIVITY QUARTERLY REPORT

(SAMPLE)

Unit Name
AFSOC Operational Medical Unit Quarterly Executive Summary X Quarter CYXX

Flight Commander's/ Element OIC's Overview Completed Deployments/Exercises (UNCLASS)
List dates, supported mission/name of exercise (if not classified), UTCs

Upcoming Deployments/Exercises
(UNCLASS) Logistics
Personnel Training
Clinical: (See sections 2.3, 2.4 and 4.1 for additional information on requirements)

Accountability for pharmacologic management of Operational Fatigue Countermeasures.
(number of patients, number of doses dispensed, number of doses returned)

Not-Mission Qualified/Not Mission Ready and Mission Ready Clinical Currency in tables on following page

Current Issues

//SIGNED//
SOW/SG or Senior Flight Surgeon

Attachment 5

AFSOC OPERATIONAL MEDICAL PERSONNEL TRAINING REQUIREMENT WAIVER REQUEST

Figure A5.1. AFSOC Operational Medical Personnel Training Requirement Waiver Request.

AFSOC OPERATIONAL MEDICAL PERSONNEL TRAINING REQUIREMENT WAIVER REQUEST

(SAMPLE)

(Unit letterhead)

Date

MEMORANDUM FOR SQUADRON/CC
GROUP/CC (as applicable)
AFSOC/SGXT
AFSOC/SGP
AFSOC/SG
IN TURN

FROM: XX SOW/SG
Mailing Address

SUBJECT: AFSOCI 48-1010 Waiver Request

Waivers to AFSOCI 48-1010 must include:

Paragraph number and name defining requirement to be waived. Reason for inability to comply with requirement.

If waiver is a follow-on to a previous waiver, explain why another waiver is required.

Description of the plan to attain compliance.

Date of expected compliance.

SOW/SG (or equivalent officer) from the originating unit must sign AFSOCI 48- 1010 waiver requests and forward it to AFSOC/SG (waivers may be submitted electronically by email).

Approved waivers will expire at expected compliance date or 90 days from approval date, whichever occurs first.

Signature Block

SOW/SG

Attachment 6

AFTER ACTION REPORT TEMPLATE

Figure A6.1. After Action Report Template Part 1.

AFTER ACTION REPORT TEMPLATE

DEPLOYMENT, IW/MSO, EXERCISES, SITE SURVEY – AAR GUIDE

Overall Classification (Secret, Confidential, Unclassified)
*****If something does not apply, put N/A and move on*****

MEMORANDUM FOR UNIT/GROUP CC
OTHERS (as your unit requires)

FROM: UNIT/INDIVIDUAL

SUBJECT: (CLASSIFICATION) Deployment After Action Report (AAR) for Trip Report, Exercise name/Deployment Name/Mission Name

DERIVED FROM: Single Source [Cite the Title and Date of Source Document] or Multiple Sources **DECLASSIFY ON:** List the declassify date from the single source, or the most stringent for multiple sources or add 25 years to the date the document was created.

**note: Each paragraph needs to have its own classification, S=Secret, U=Unclassified, C=Confidential
– If this is a classified report, type, update and submit in SIPR.

1. (CLASSIFICATION) **NAME AND LOCATION:** Update this information

2. (CLASSIFICATION) **INCLUSIVE DATES:** 30 Jan – 27 Feb 20XX

2.1. (CLASSIFICATION) **PREDEPLOYMENT TRAINING:** Attended T1G training from DD MMM to DD MMM, LTT at DD MMM to DD MMM, etc.

2.2. (CLASSIFICATION) **DAYS OF TRAVEL TO LOCATION:** Travel from CONUS to Location took 4 days, plane delay at LOCATION, arrived at Base X on DD MMM YY.

2.3. (CLASSIFICATION) **DAYS AT DEPLOYED LOCATION (AND OTHER TRAVEL**

LOCATIONS/DATES): 45 days at deployed site, forward deployed to FOB XYZ, etc.

2.4. (CLASSIFICATION) **REDEPLOYMENT TRAVEL:** Redeployed via space-a on helicopter from place Y to Place Z. Manifested on rotator from Base ZYX to CONUS. Arrived home station on DD MMM YY.

3. (CLASSIFICATION) **MISSION OBJECTIVE(S), CONOPS AND UNIT(S) SUPPORTED:** These were our mission objectives, our CONOPS were and we supported the 123 unit while they did blank missions.

3.1. (CLASSIFICATION) Expound as necessary especially for multiple missions objectives, CONOPS etc.

3.2. (CLASSIFICATION)

3.3. (CLASSIFICATION)

4. (CLASSIFICATION) **PERSONNEL AND EQUIPMENT**

Figure A6.2. After Action Report Template Part 2.

4.1. (CLASSIFICATION) **MISSION COMMANDER:** Col YYY was mission commander, medical commander was Maj YYY.

4.2. (CLASSIFICATION) **NUMBER OF DEPLOYED PERSONNEL:** Total PAR was ## (## AD, ## Contractors, ## Local Nationals/TCNs)

4.3. (CLASSIFICATION) **Personnel in Reporting Set:** 1 48R3, 1 – 4N071C

5. (CLASSIFICATION) **OPERATIONS OVERVIEW:** We performed ABC, supporting XYZ. On stand-by for CASEVAC missions.

5.1. (CLASSIFICATION) **TOTAL MISSION FLYING TIME AND SORTIES FLOWN**: 230 hours total mission flying time, 95 sorties flown.

5.2. (CLASSIFICATION) **FLYING TME AND SORTIES FLOWN BY MEDICAL PERSONNEL:** 120 hours total mission flying time, 30 sorties flown.

5.3. (CLASSIFICATION) **PATIENTS TREATED DURING FLYING:** Summary of patients treated during flight. Reference appropriate AARs already completed do not duplicate what has already been reported via CASEVAC AARs etc.

6. (CLASSIFICATION) **MISSION ESSENTIAL TASK LIST (METL) EVENTS ACCOMPLISHED (to include AFSOCI 48-101 REQUIREMENTS):** What METLs were completed, what was accomplished IAW AFSOCI 48-1010 requirements, you should get credit for.

7. (CLASSIFICATION) **HOST NATION SUPPORT:**

7.1. (CLASSIFICATION) **LODGING:** Lodging provided by FSS while at base XYZ. Lodging safety, sanitation, proximity, quality, etc. during forward operations was…

7.2. (CLASSIFICATION) **MEDICAL:** What type of host nation medical facilities were available, NATO/Military Treatment Facilities…

7.3. (CLASSIFICATION) **FOOD/WATER SUPPLIES:** Any specific, good to know information for someone who is going to be going to that location.

8. (CLASSIFICATION) **TRANSPORTATION:**

8.1. (CLASSIFICATION) **MEDICAL TEAM TRANSPORTATION:** What type of vehicles (if any), airframe used, local national ambulances etc.

8.2. (CLASSIFICATION) **PATIENT TRANSPORTATION:** How did patients get to you, how did they get to definitive care whether via AE, host-nation ambulance to a hospital.

9. (CLASSIFICATION) **COMMUNICATION:** What communication challenges did you experience? What did you have available. What would have been good to have. NIPR and/or SIPR? Satellite phones etc.

10. (CLASSIFICATION) **FACILITIES:**

10.1. (CLASSIFICATION) **BILLETING:** Was lodging available, did you stay in a building of opportunity, what would have been good to know prior to leaving or what changed while you were there.

10.2. (CLASSIFICATION) **OPERATIONS FACILITY:** Where did you work out of? Was there a hardened building/facility etc.

Figure A6.3. After Action Report Template Part 3.

11. (CLASSIFICATION) **MEDICAL SPECIFICS:**

11.1. (CLASSIFICATION) **WATER SUPPLY:** Bottled water supplied by a contractor? Used self-brought water purification system.

11.2. (CLASSIFICATION) **SEWER AND DISPOSAL:** Burn pits? Trailers managed by a contractor?

11.3. (CLASSIFICATION) **LOCAL RESTURANTS: Safety, sanitation, proximity, quality, etc.** On base and off base if applicable.

11.4. (CLASSIFICATION) **INSECTS AND ANIMALS:** What do individuals need to be aware of.

11.5. (CLASSIFICATION) **PATIENTS WORKLOAD/PAR:** What was the population at risk?

11.5.1. (CLASSIFICATION) **TOTAL PATIENT ENCOUNTERS:** Total patients treated by you/your team

11.5.2. (CLASSIFICATION) **MINOR PROCEDURES:** How many and what kind.

11.5.3. (CLASSIFICATION) **FLYING MISSION:** What was the flying mission, how frequent?

11.5.4. (CLASSIFICATION) **OTHER MEDICAL MISSIONS**: Other medical missions, i.e. FOB build up, host-nation education etc.

11.5.5. (CLASSIFICATION) **MEDICAL EVACUATIONS:** How many? What kind, etc.

11.5.6. (CLASSIFICATION) **PATIENTS ROTATED HOME EARLY FOR MEDICAL REASONS:** Short synopsis of why and how many.

12. (CLASSIFICATION) **SAFETY:** Safety concerns, environmental? Equipment?

13. (CLASSIFICATION) **MEDICAL MODERNIZATION OPPORTUNITIES:** Use this section to identify equipment needs, problems or shortcomings. Include ideas or observations where improved capabilities could have been improved: mission or task effectiveness, consistent delivery of patient care, safety, quality of life, time or manpower savings, risk mitigation or cost savings. Please describe as much detail as possible of the idea and potential benefits. Do not use this section to address logistical shortfalls unless they may be addressed through a research and/or developmental effort.

14. (CLASSIFICATION) **TURNOVER/END OF DEPLOYMENT:** Challenges faced while waiting on replacement. Was overlap time appropriate for familiarization to mission and coordination, etc.

15. (CLASSIFICATION) **LESSONS LEARNED**

15.1. (CLASSIFICATION)
Observation Item: Discussion:

Recommendation:

Is this an item for Higher HQ Action?

Figure A6.4. After Action Report Template Part 4.

15.2. (CLASSIFICATION)
Observation Item: Discussion:

Recommendation:

Is this an item for Higher HQ Action?

16. (CLASSIFICATION) SUMMARY OF MISSION: Overall summary of the mission, what went well, what could have been improved changed etc. Overall review of medical capabilities, support etc.

Signature Block

cc: AFSOC/SGX
Others as your unit requires (Clinics)

*****If something does not apply, put N/A and move on*****